

Children's Hospital Los Angeles

Consolidated Financial Statements as of and
for the Years Ended June 30, 2024 and 2023, and
Schedule of Expenditures of Federal Awards, and
OMB Uniform Grant Guidance Compliance Reports
for the Year Ended June 30, 2024, and
Independent Auditor's Reports

CHILDREN'S HOSPITAL LOS ANGELES

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INDEPENDENT AUDITOR'S REPORT

To the Board of Directors of
Children's Hospital Los Angeles:

Report on the Audit of the Financial Statements

Opinion

We have audited the accompanying consolidated financial statements of Children's Hospital Los Angeles and its subsidiaries (the "Hospital"), which comprise the consolidated balance sheets as of June 30, 2024 and 2023, and the related consolidated statements of activities, changes in net assets, and cash flows for the years then ended, and the related notes to the consolidated financial statements (collectively referred to as the "financial statements").

In our opinion, the accompanying financial statements present fairly, in all material respects, the consolidated financial position of the Hospital as of June 30, 2024 and 2023, and the results of its operations, changes in its net assets, and its cash flows for the years then ended in accordance with accounting principles generally accepted in the United States of America.

Basis for Opinion

We conducted our audits in accordance with auditing standards generally accepted in the United States of America (GAAS) and the standards applicable to financial audits contained in *Government Auditing Standards*, issued by the Comptroller General of the United States (*Government Auditing Standards*). Our responsibilities under those standards are further described in the Auditor's Responsibilities for the Audit of the Financial Statements section of our report. We are required to be independent of the Hospital and to meet our other ethical responsibilities in accordance with the relevant ethical requirements relating to our audits. We believe that the audit evidence we have obtained is sufficient and appropriate to provide a basis for our audit opinion.

Responsibilities of Management for the Financial Statements

Management is responsible for the preparation and fair presentation of the financial statements in accordance with accounting principles generally accepted in the United States of America and for the design, implementation, and maintenance of internal control relevant to the preparation and fair presentation of financial statements that are free from material misstatement, whether due to fraud or error.

In preparing the financial statements, management is required to evaluate whether there are conditions or events, considered in the aggregate, that raise substantial doubt about the Hospital's ability to continue as a going concern for one year after the date that the financial statements are issued.

Auditor's Responsibilities for the Audit of the Financial Statements

Our objectives are to obtain reasonable assurance about whether the financial statements as a whole are free from material misstatement, whether due to fraud or error, and to issue an auditor's report

that includes our opinion. Reasonable assurance is a high level of assurance but is not absolute assurance and therefore is not a guarantee that an audit conducted in accordance with GAAS and *Government Auditing Standards* will always detect a material misstatement when it exists. The risk of not detecting a material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control. Misstatements are considered material if there is a substantial likelihood that, individually or in the aggregate, they would influence the judgment made by a reasonable user based on the financial statements.

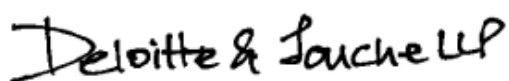
In performing an audit in accordance with GAAS and *Government Auditing Standards*, we:

- exercise professional judgment and maintain professional skepticism throughout the audit.
- identify and assess the risks of material misstatement of the financial statements, whether due to fraud or error, and design and perform audit procedures responsive to those risks. Such procedures include examining, on a test basis, evidence regarding the amounts and disclosures in the financial statements.
- obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of the Hospital's internal control. Accordingly, no such opinion is expressed.
- evaluate the appropriateness of accounting policies used and the reasonableness of significant accounting estimates made by management, as well as evaluate the overall presentation of the financial statements.
- conclude whether, in our judgment, there are conditions or events, considered in the aggregate, that raise substantial doubt about the Hospital's ability to continue as a going concern for a reasonable period of time.

We are required to communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit, significant audit findings, and certain internal control-related matters that we identified during the audit.

Other Reporting Required by *Government Auditing Standards*

In accordance with *Government Auditing Standards*, we have also issued our report dated November 18, 2024, on our consideration of the Hospital's internal control over financial reporting and on our tests of its compliance with certain provisions of laws, regulations, contracts, and grant agreements and other matters. The purpose of that report is to describe the scope of our testing of internal control over financial reporting and compliance and the results of that testing, and not to provide an opinion on the effectiveness of the Hospital's internal control over financial reporting or on compliance. That report is an integral part of an audit performed in accordance with *Government Auditing Standards* in considering the Hospital's internal control over financial reporting and compliance.



November 18, 2024

CHILDREN'S HOSPITAL LOS ANGELES

CONSOLIDATED BALANCE SHEETS AS OF JUNE 30, 2024 AND 2023 (In thousands)

	2024	2023
ASSETS		
CURRENT ASSETS:		
Cash and cash equivalents	\$ 106,006	\$ 41,225
Patient accounts receivable (Note 1)	309,794	226,106
Contract assets (Note 3)	72,072	58,485
Current portion of pledges receivable—net (Note 8)	35,953	34,421
Grants receivable	36,820	22,089
Receivables under government and state programs (Note 5)	41,329	29,438
Current portion of trustee-held funds (Notes 6, 7, and 9)	7,631	7,503
Hospital Fee Program receivables (Note 19)	283,591	293,956
Other current assets	44,172	38,169
Total current assets	937,368	751,392
ASSETS LIMITED AS TO USE (Notes 6 and 7):		
Investments	640,193	840,843
Unitrust investments	6,419	6,051
Total assets limited as to use—net of current portion	646,612	846,894
PLEDGES RECEIVABLE—Net of current portion—net (Note 8)	24,105	27,287
OTHER ASSETS (Notes 1 and 20)	72,043	66,603
PROPERTY, PLANT, AND EQUIPMENT—Net (Note 10)	988,535	1,004,777
TOTAL	\$ 2,668,663	\$ 2,696,953

(Continued)

CHILDREN'S HOSPITAL LOS ANGELES

CONSOLIDATED BALANCE SHEETS AS OF JUNE 30, 2024 AND 2023 (In thousands)

	2024	2023
LIABILITIES AND NET ASSETS		
CURRENT LIABILITIES:		
Accounts payable and accrued expenses (Note 20)	\$ 166,399	\$ 178,226
Salaries, wages, and related liabilities	101,573	89,236
Current portion of long-term debt (Note 13)	10,250	9,950
Liabilities to third-party payors (Note 11)	55,300	-
Line of credit (Note 12)	50,000	50,000
Payables under government programs (Note 5)	12,212	19,573
Hospital Fee Program payables (Note 19)	88,424	68,391
Deferred revenue and other liabilities	<u>15,658</u>	<u>15,853</u>
Total current liabilities	499,816	431,229
LONG-TERM DEBT—Net of current portion (Notes 1 and 13)	421,995	432,902
LIABILITY UNDER UNITRUST AGREEMENTS	1,653	1,723
INTEREST RATE SWAP (Note 13)	2,774	3,896
OTHER NONCURRENT LIABILITIES (Notes 5, 14, and 20)	<u>55,417</u>	<u>85,246</u>
Total liabilities	<u>981,655</u>	<u>954,996</u>
COMMITMENTS AND CONTINGENCIES (Note 14)		
NET ASSETS:		
Without donor restrictions	1,094,112	1,143,085
With donor restrictions (Note 15)	<u>592,896</u>	<u>598,872</u>
Total net assets	<u>1,687,008</u>	<u>1,741,957</u>
TOTAL	<u>\$ 2,668,663</u>	<u>\$ 2,696,953</u>

See notes to consolidated financial statements.

(Concluded)

CHILDREN'S HOSPITAL LOS ANGELES

CONSOLIDATED STATEMENTS OF ACTIVITIES FOR THE YEARS ENDED JUNE 30, 2024 AND 2023 (In thousands)

	2024	2023
REVENUES (Note 4):		
Patient service revenue (Note 3)	\$ 994,045	\$ 871,526
Hospital Fee Program revenue (Note 19)	348,241	336,821
Net patient service revenue (Note 3)	1,342,286	1,208,347
Grants, contracts, and other	227,634	188,227
Unrestricted gifts and bequests	47,249	80,510
Investment income used for operations, research, and education	20,646	15,564
Net assets released from restrictions used for operations, research, and education	102,925	90,363
Total revenues	1,740,740	1,583,011
EXPENSES (Note 17):		
Salaries and employee benefits (Note 16)	970,094	879,864
Professional fees and purchased services	313,974	304,927
Supplies	265,925	223,799
Hospital Fee Program (Note 19)	77,274	62,631
Other (Note 20)	91,760	92,360
Total expenses before depreciation, amortization, and interest	1,719,027	1,563,581
EXCESS OF REVENUES OVER EXPENSES BEFORE DEPRECIATION, AMORTIZATION, AND INTEREST	21,713	19,430
DEPRECIATION, AMORTIZATION, AND INTEREST:		
Depreciation and amortization	78,063	69,680
Interest	21,322	19,685
Total depreciation, amortization, and interest	99,385	89,365
DEFICIT OF REVENUES OVER EXPENSES	(77,672)	(69,935)
OTHER GAINS:		
Other investment income—net of fees (Note 6)	34,246	36,865
Net investment income used for operations, research, and education	(20,646)	(15,564)
Interest rate swap mark-to-market gain (Note 13)	1,122	2,700
Other gains	-	1,696
Total other gains	14,722	25,697
DEFICIT OF REVENUES OVER EXPENSES AND OTHER GAINS	\$ (62,950)	\$ (44,238)

See notes to consolidated financial statements.

CHILDREN'S HOSPITAL LOS ANGELES

CONSOLIDATED STATEMENTS OF CHANGES IN NET ASSETS FOR THE YEARS ENDED JUNE 30, 2024 AND 2023 (In thousands)

	2024	2023
NET ASSETS WITHOUT DONOR RESTRICTIONS:		
Deficit of revenues over expenses and other gains	\$ (62,950)	\$ (44,238)
Proposition 4—Building construction advances	10,647	-
Net assets released from restrictions used for purchase of property and equipment	10,089	15,231
Transfers and other	<u>(6,759)</u>	<u>(3,687)</u>
Decrease in net assets without donor restrictions	<u>(48,973)</u>	<u>(32,694)</u>
NET ASSETS WITH DONOR RESTRICTIONS:		
Restricted grants, gifts, and bequests	58,625	99,206
Provision for uncollectible pledges (Note 8)	1,040	760
Net investment income on restricted gifts and endowments (Note 6)	9,367	16,239
Net realized and unrealized gain restricted on investments (Note 6)	32,580	26,419
Net assets released from restrictions used for operations, research, and education	(102,925)	(90,363)
Net assets released from restrictions used for purchase of property and equipment	(10,089)	(15,231)
Transfers and other	5,591	1,461
Change in value of split-interest agreements	<u>(165)</u>	<u>537</u>
(Decrease) Increase in net assets with donor restrictions	<u>(5,976)</u>	<u>39,028</u>
CHANGE IN NET ASSETS	(54,949)	6,334
NET ASSETS—Beginning of year	<u>1,741,957</u>	<u>1,735,623</u>
NET ASSETS—End of year	<u>\$ 1,687,008</u>	<u>\$ 1,741,957</u>

See notes to consolidated financial statements.

CHILDREN'S HOSPITAL LOS ANGELES

CONSOLIDATED STATEMENTS OF CASH FLOWS FOR THE YEARS ENDED JUNE 30, 2024 AND 2023 (In thousands)

	2024	2023
CASH FLOWS FROM OPERATING ACTIVITIES:		
Change in net assets	\$ (54,949)	\$ 6,334
Adjustments to reconcile change in net assets to net cash used in operating activities:		
Gain on early extinguishment of debt	-	(1,717)
Depreciation of property, plant, and equipment	77,774	69,373
Noncash change to right-of-use asset	10,937	8,734
Amortization of bond cost, discount, and premium	(657)	(741)
Net realized and unrealized gain on investments	(60,177)	(50,307)
Proposition 4—building construction advances	(10,647)	-
Loss on disposal of property and equipment—net	450	21
Contributions restricted for purchases of long-lived assets	(12,387)	(12,062)
Restricted contributions and other changes	(34,890)	(22,901)
Interest rate swap mark-to-market gain	(1,122)	(2,700)
Changes in operating assets and liabilities:		
Patient accounts receivable and contract assets	(97,275)	(39,443)
Pledges receivable—net	1,650	30,860
Grants receivable	(14,731)	(2,917)
Hospital Fee Program receivables	10,365	(124,538)
Other current assets	(6,003)	(1,389)
Other assets	(16,744)	(9,526)
Accounts payable and accrued expenses	(10,544)	62,522
Salaries, wages, and related liabilities	12,337	(17,005)
Payables under government and state programs	(19,252)	(10,257)
Hospital Fee Program payables	20,033	16,133
Deferred revenue	(195)	(3,659)
Other liabilities	(18,341)	1,383
Operating lease liabilities	(11,363)	(8,471)
Net cash used in operating activities	<u>(235,731)</u>	<u>(112,273)</u>
CASH FLOWS FROM INVESTING ACTIVITIES:		
Purchase of investments	(91,487)	(183,469)
Sale of investments and maturities of debt securities	352,314	244,247
Cost of property, plant, and equipment acquired	(63,461)	(56,446)
Transfers from trustee-held funds	-	(397)
Net cash provided by investing activities	<u>197,366</u>	<u>3,935</u>
CASH FLOWS FROM FINANCING ACTIVITIES:		
Payments on long-term debt	(9,950)	-
Advanced refunding of long-term debt	-	(110,795)
Proceeds from issuance of long-term debt	-	103,420
Payment of financing costs	-	(789)
Contributions restricted for purchase of long-lived assets	12,387	12,062
Proposition 4—building construction advances	10,647	-
Proceeds from third-party payors	55,300	-
Proceeds from line of credit	-	50,000
Proceeds from restricted contributions and other changes	34,890	22,901
Net cash provided by financing activities	<u>103,274</u>	<u>76,799</u>
INCREASE (DECREASE) IN CASH, CASH EQUIVALENTS, AND RESTRICTED CASH	64,909	(31,539)
CASH, CASH EQUIVALENTS, AND RESTRICTED CASH—Beginning of year	48,728	80,267
CASH, CASH EQUIVALENTS, AND RESTRICTED CASH—End of year	<u>\$ 113,637</u>	<u>\$ 48,728</u>
SUPPLEMENTAL INFORMATION RELATING TO NONCASH ITEMS—Property and equipment acquisitions included in accounts payable	<u>\$ 2,103</u>	<u>\$ 3,582</u>

See notes to consolidated financial statements.

CHILDREN’S HOSPITAL LOS ANGELES

NOTES TO CONSOLIDATED FINANCIAL STATEMENTS

AS OF AND FOR THE YEARS ENDED JUNE 30, 2024 AND 2023

1. ORGANIZATION AND SUMMARY OF SIGNIFICANT ACCOUNTING POLICIES

Organization—Children’s Hospital Los Angeles (CHLA or the “Hospital”), a not-for-profit (NFP) corporation organized under the laws of the State of California, is a tax-exempt organization.

CHLA is the direct controlling entity of CHLA Holdings, LLC; Children’s Health System of Los Angeles, LLC; CHLA Health Network, LLC; CHLA Medical Foundation; CHLA International, LLC; CHLA Teaching Clinics, LLC; CHLA Electing Small Business Trust (“CHLA ESBT”); CHLA Foundation, LLC; 4520 Sunset Blvd, LLC; 4470 Sunset Blvd, LLC; 4455 De Longpre Ave, LLC; and 1520 Rodney Dr, LLC. The purpose of CHLA Holdings, LLC is to hold title to property, collect income therefrom, and turn over the entire amount thereof, less expenses, to CHLA. The purpose of Children’s Health System of Los Angeles, LLC is to provide or arrange for the provision of health care services and educational and research activities associated with such health care services by CHLA and, to date, has had no activity. The purpose of CHLA Health Network, LLC is to manage, coordinate, and demonstrate high-value patient- and family-centered pediatric care through the clinical integration of CHLA and the physicians on the medical staff of CHLA and in CHLA’s market.

The purpose of CHLA International, LLC is to further develop the international health care programs of CHLA. The purpose of CHLA Teaching Clinics, LLC is to provide, manage, coordinate, and enhance the quality of pediatric patient care through the operation of multiple teaching clinics that are exempt from licensure under Section 1206(g) of the California Health and Safety Code. CHLA ESBT was set up for retaining Helms-Howard Inc. stock. CHLA has 8.33% ownership interest in Helms-Howard Inc.

CHLA has a controlling interest in Virtual Pediatric Intensive Care Unit Systems, LLC (VPS), owning 98% of VPS. The purpose of VPS is to provide quality reports utilizing a clinical database dedicated to standardized data sharing and benchmarking among pediatric intensive care units.

CHLA Medical Foundation is a nonpublic benefit corporation and is organized and operated exclusively for charitable purposes and, to date, has had no activity.

CHLA is the sole statutory member of the CHLA Foundation, LLC. CHLA Foundation, LLC’s purpose includes fund-raising on behalf of the Hospital and collaborating with the Hospital’s management in determining its activities in support of its fund-raising. It is not intended to directly receive donations from the public; however, any donations received by the CHLA Foundation, LLC shall be transferred to the Hospital at the sole and absolute discretion of the Hospital and, to date, has had no activity.

CHLA is the sole member of 4520 Sunset Blvd, LLC. The purpose of 4520 Sunset Blvd, LLC is to hold certain real estate properties for the future expansion and development of CHLA.

CHLA is the sole member of 4470 Sunset Blvd, LLC. The purpose of 4470 Sunset Blvd, LLC is to hold certain real estate properties for the future expansion and development of CHLA.

CHLA is the sole member of 4455 De Longpre Ave, LLC. The purpose of 4455 De Longpre Ave, LLC is to hold certain real estate properties for the future expansion and development of CHLA.

CHLA is the sole member of 1520 Rodney Dr, LLC. The purpose of 1520 Rodney Dr, LLC is to hold certain real estate properties for the future expansion and development of CHLA.

The Hospital's principal mission is to promote and advance the state of children's health, focusing on tertiary and quaternary specialties in patient care, research, and education.

Basis of Presentation—The accompanying consolidated financial statements and accompanying notes have been prepared in accordance with accounting principles generally accepted in the United States of America (US GAAP). The consolidated financial statements include the accounts of the Hospital, subsidiaries, entities where the Hospital has a controlling financial interest, and immaterial variable interest entities in which the Hospital is the primary beneficiary. All intercompany transactions have been eliminated in consolidation.

Use of Estimates—The preparation of consolidated financial statements in conformity with US GAAP requires management to make estimates and assumptions that affect the reported amounts of assets and liabilities and disclosure of contingent assets and liabilities at the date of the consolidated financial statements. Estimates also affect the reported amounts of revenues and expenses during the reporting period. Actual results could differ from those estimates.

Deficit of Revenues over Expenses and Other Gains—Management considers deficit of revenues over expenses and other gains to be the Hospital's performance indicator. Consistent with industry practice, changes in net assets without donor restrictions excluded from this total would include any revenues recognized from permanent transfers of assets to and from affiliates for other than goods and services, contributions of long-lived assets (including assets acquired using contributions that by donor restriction were to be used for the purposes of acquiring such assets), and extraordinary funds received directly from public sources.

Net Patient Service Revenue—Net patient service revenue is reported at the estimated net realizable amounts from patients; third-party payors; and others, including estimated variable consideration for retroactive adjustments due to terms under reimbursement agreements with third-party payors, settlement of audits, reviews, and investigations. Retroactive adjustments are estimated and accrued in the period in which the related services are rendered and adjusted in future periods as final settlements are determined.

The Hospital determines performance obligations based on the nature of the services provided. Patient service revenue is recognized for performance obligations satisfied over time based on actual charges incurred in relation to total expected charges. The Hospital believes that this method provides a faithful depiction of the transfer of services over the term of performance obligations based on the inputs needed to satisfy the obligations. Generally, performance obligations satisfied over time relate to patients in the Hospital receiving inpatient acute care services. Performance obligations are measured from admission to the point when there are no further services required for the patient, which is generally the time of discharge. Revenue is recognized for performance obligations satisfied at a point in time, which generally relate to patients receiving outpatient services, when (1) services are provided and (2) the Hospital does not believe the patient requires additional services. Because the patient service performance obligations relate to contracts with a duration of less than one year, the Hospital has elected to apply the optional exemption provided in Financial Accounting Standards Board (FASB) Accounting Standards Codification (ASC) 606-10-50-

14(a), *Revenue from Contracts with Customers (Topic 606)*, and, therefore, the Hospital is not required to disclose the aggregate amount of the transaction price allocated to performance obligations that are unsatisfied or partially unsatisfied at the end of the reporting period. The unsatisfied or partially unsatisfied performance obligations referred to above are primarily related to inpatient acute care services at the end of the reporting period. The performance obligations for these contracts are generally completed when the patients are discharged, which generally occurs within days or weeks of the end of the reporting period, except for certain patients who may have much longer lengths of stay.

The Hospital determines the transaction price, which involves significant estimates and judgment, based on gross charges utilizing standard billing rates for services provided, reduced by contractual adjustments provided to third-party payers, discounts provided to uninsured patients, and implicit price concessions provided primarily to uninsured or underinsured patients. The Hospital determines its estimates of contractual adjustments and discounts based on contractual agreements, its discount policy, and historical experience. The Hospital determines its estimate of implicit price concessions based on historical collection experience with these classes of patients using a portfolio approach as a practical expedient to account for patient contracts as collective groups rather than on an individual contract basis. The consolidated financial statement effects of using this practical expedient are not materially different from an individual contract approach.

Revenues under the traditional fee-for-service Medicaid programs, including California's Medicaid program ("Medi-Cal"), are based primarily on prospective payment systems, plus an outlier payment for individual patients whose medical conditions are costlier to treat than those of patients with the same diagnosis.

Settlements with third-party payers for retroactive revenue adjustments due to audits, reviews, or investigations are considered variable consideration and are included in the determination of the estimated transaction price for providing patient care utilizing the most likely outcome method. These settlements are estimated based on the terms of the payment agreement with the payer, correspondence from the payer, and historical settlement activity, including an assessment to ensure that it is probable that a significant reversal in the amount of cumulative revenue recognized will not occur when the uncertainty associated with the retroactive adjustment is subsequently resolved. Estimated settlements are adjusted in future periods as adjustments become known (that is, new information becomes available), or as years are settled or are no longer subject to such audits, reviews, or investigations.

Revenues under managed care plans are based primarily on payment terms involving predetermined rates per diagnosis, per diem rates, discounted fee-for-service rates, and/or other similar contractual arrangements. These revenues are also subject to review and possible audit by the payers, which can take several years before they are completely resolved. The payers are billed for patient services on an individual patient basis. An individual patient's bill is subject to adjustment on a patient-by-patient basis in the ordinary course of business by the payers following their review and adjudication of each particular bill. The Hospital estimates the discounts for contractual allowances. At the end of each month, the Hospital estimates its expected reimbursement for patients of managed care plans based on the applicable contract terms. Contractual allowance estimates are periodically reviewed for accuracy by taking into consideration known contract terms, as well as payment history. The Hospital believes that the estimation and review process enables them to identify instances on a timely basis where such estimates need to

be revised. The Hospital does not believe there were any adjustments to estimates of patient bills that were material to revenues.

Generally, patients who are covered by third-party payers are responsible for related copays, coinsurance, and deductibles, which vary in amount. The Hospital also provides services to uninsured patients and offers uninsured patients a discount from standard charges. The Hospital estimates the transaction price for patients with copays, coinsurance, and deductibles and for those who are uninsured based on historical collection experience and current market conditions. Under the Hospital's uninsured discount programs, the discount offered to certain uninsured patients is recognized as a policy discount, which reduces net operating revenues at the time the self-pay accounts are recorded. The uninsured patient accounts, net of policy discounts recorded, are further reduced to their net realizable value at the time they are recorded through implicit price concessions based on historical collection trends for self-pay accounts and other factors that affect the estimation process. Subsequent changes to the estimate of the transaction price are generally recorded as adjustments to net patient service revenues in the period of the change. Although outcomes vary, the Hospital's policy is to attempt to collect amounts due from patients, including copays, coinsurance, and deductibles due from patients with insurance, at the time of service while complying with all federal and state statutes and regulations.

Cash and Cash Equivalents—Cash and cash equivalents, including highly liquid instruments with original maturities of three months or less at the date of purchase, are recorded at cost, which approximates market value. The Hospital holds deposits in excess of Federal Deposit Insurance Corporation limits. Uninsured and uncollateralized deposits were approximately \$104.2 million and \$39.7 million at June 30, 2024 and 2023, respectively. These deposits are held by creditworthy, high-quality financial institutions.

Restricted Cash—Current restricted cash is related to trustee-held funds for debt service (see Note 9) and noncurrent restricted cash is related to collateral for interest rate swaps (see Note 13).

The following table provides a reconciliation of cash, cash equivalents, and restricted cash reported in the consolidated balance sheets that sum to amounts reported in the consolidated statements of cash flows as of June 30, 2024 and 2023 (in thousands):

	2024	2023
Cash and cash equivalents	\$ 106,006	\$ 41,225
Restricted cash—current portion of trustee-held funds	<u>7,631</u>	<u>7,503</u>
Total cash, cash equivalents, and restricted cash	<u>\$ 113,637</u>	<u>\$ 48,728</u>

Patient Accounts Receivable—Patient accounts receivable are recorded on an accrual basis at net realizable value upon billing for services provided to patients.

Contract Assets—Amounts related to services provided to patients for which the Hospital has not yet billed and that do not meet the conditions of unconditional right to payment at the end of the reporting period are contract assets. The Hospital's contract assets consist primarily of services provided to patients who are still receiving inpatient care in the Hospital's facilities at the end of the reporting period at estimated net realizable value.

Pledges Receivable—The Hospital accounts for pledges in accordance with ASC 958-605, *Not-For-Profit Entities—Revenue Recognition*, amended by Accounting Standards Update (ASU) No. 2018-08, *Not-For-Profit Entities (Topic 958): Clarifying the Scope and the Accounting Guidance for*

Contributions Received and Contributions Made. Pledges due within 12 months are included in current assets, and pledges due after 12 months are included in noncurrent assets. The pledges due beyond one year are reported at their net present value using risk-free interest rates at the date of such pledges. For the year 2024, the discount rates in effect on outstanding pledges receivable varied between 0.28% and 5.51%. For the year 2023, the discount rates in effect on outstanding pledges receivable varied between 0.16% and 4.87%. Pledges are reported net of an estimated allowance for uncollectible amounts.

Inventories—Inventories are stated at the lower of cost or net realizable value, which is determined utilizing the weighted-average method and are included in other current assets in the accompanying consolidated balance sheets. The carrying value of inventories is approximately \$21 million and \$16.2 million as of June 30, 2024 and 2023, respectively.

Property, Plant, and Equipment—Property, plant, and equipment acquired by purchase are recorded at cost. Donated items are recorded at estimated fair value at the date of donation. Depreciation is computed on a straight-line basis over the estimated useful lives of the depreciable assets. Estimated useful lives by classification are as follows:

Land improvements	2–20 years
Buildings and improvements	5–40 years
Equipment	3–20 years
Computer equipment	3–5 years

Leases—The Hospital adopted FASB ASC 842, *Leases*, which required leases with durations longer than a year to be recognized in the consolidated balance sheets. The Hospital made accounting policy elections, including a short-term lease exception policy, permitting the Hospital to not apply the recognition requirements of this standard to short-term leases (i.e., leases with expected terms of 12 months or less) and an accounting policy to account for lease and certain nonlease components as a single component for certain classes of assets.

The Hospital recognizes right-of-use (ROU) assets and lease liabilities for leases with terms greater than 12 months or leases that contain a purchase option that is reasonably certain to be exercised. Leases are classified as either finance or operating leases. The lease expense is recognized based on an effective interest method or on a straight-line method over the term of the lease. The Hospital's ROU assets and liabilities primarily relate to real estate leases for physician practices, office space, patient service areas, health care equipment, and office equipment. Certain facility leases include one or more options to renew. The exercise of lease renewal options is within the Hospital's control. None of the Hospital's lease agreements contain material residual value guarantees (see Note 20).

Asset Impairment—In accordance with FASB ASC 360, *Property, Plant, and Equipment*, the Hospital evaluates long-lived assets for possible impairment annually or whenever events or changes in circumstances indicate that the carrying amount of the asset, or related group of assets, may not be recoverable from estimated future undiscounted cash flows. If the estimated future undiscounted cash flows are less than the carrying value of the assets, the Hospital calculates the amount of an impairment if the carrying value of the long-lived assets exceeds the fair value of the assets. No asset impairments were recorded during the years ended June 30, 2024 and 2023.

Assets Limited as to Use—Assets limited to use relate to investments of assets restricted by donors or designated by the board of directors of CHLA (the "Board of Directors") for future capital improvements, patient care, research, and other uses. The net assets over which the Board of

Directors retains control may subsequently be used for other purposes at the Board of Directors' discretion. Assets held by trustees under indenture agreements, escrow deposits, and various unitrusts are also included within assets limited to use, except for the current portion of any trustee-held funds, which are classified separately.

Investments consist primarily of equity and debt securities purchased by the Hospital and unitrust accounts, for which the Hospital has been designated as trustee and beneficiary. Investments in equity securities with readily determinable fair values and all investments in debt securities are measured at fair value based upon publicly quoted market prices or quotations of similar securities (see Note 7). Investments for which readily determinable market values do not exist are recorded based upon the net asset value (NAV) per share of the underlying fund, determined by the Hospital, with the assistance of fund managers, the general partners, or third-party service providers, utilizing methods and significant assumptions the Hospital considers appropriate based on its understanding of the underlying characteristics of the investments. These types of investments include private investment funds and commingled funds.

The Hospital classifies its investments in debt securities as trading and, as such, all net unrealized holding gains or losses without donor restrictions are recorded in other gains in the period in which they occur. All net unrealized holding gains or losses without donor restrictions are also recorded in other gains in the period in which they occur in accordance with ASU No. 2016-01, *Financial Instruments—Overall (Subtopic 825-10): Recognition and Measurement of Financial Assets and Financial Liabilities*.

Investment income (which includes interest and dividends) and realized and unrealized gains and losses are included in the deficit of revenues over expenses and other gains, unless the income or gain (loss) is restricted by the donor or law. Such restricted investment income is included in the increase in net assets with donor restrictions in the consolidated statements of changes in net assets.

Investment securities, in general, are exposed to various risks, such as interest rate, credit, and overall market volatility. Due to the level of risk associated with certain investment securities, it is reasonably possible that changes in the values of investment securities will occur in the near term and that such changes could materially affect the amounts reported in the accompanying consolidated balance sheets.

Hospital Fee Program—In January 2010, the State of California enacted legislation that provides for supplemental Medi-Cal payments to certain hospitals funded by a quality assurance fee (QAF) paid by participating hospitals as well as matching federal funds (the "Hospital Fee Program"). Funds received and fees paid under the Hospital Fee Program are recognized in the deficit of revenues over expenses and other gains (losses) at the inception of the program based on modeling that is part of the program approval submission that has been provided to hospitals by the state hospital association. Funds or fees not received or paid based on the model are included in the consolidated balance sheets as Hospital Fee Program receivables and Hospital Fee Program payables, respectively (see Note 19).

Proposition 52 in November 2016 made the Hospital Fee Program permanent in California and based on the Hospital's historical experience during the first four rounds of the program, for the fifth round (covering the period from January 1, 2017, through June 30, 2019), the Hospital recognized revenue and expenses once overall program approval was received from the Centers for Medicare and Medicaid Services (CMS). CMS granted overall approval for the fifth round of the

program in December 2017. The State of California submitted its waiver application and state plan amendment for round six of the program, which covers the period from July 1, 2019, through December 31, 2021, in October 2019. CMS granted overall approval for the sixth round of the program in February 2020. The State of California submitted a draft model to CMS for round seven of the program, which covers the period from January 1, 2022, through December 31, 2022, in December 2021. The Hospital recognized revenue and expenses at the inception of the program (January 1, 2022) based on the model distributed on December 31, 2021. CMS granted overall approval for the seventh round of the program in September 2022. The State of California submitted a draft model to CMS for round eight of the program, which covers the period from January 1, 2023, through December 31, 2024, in March 2023. The Hospital recognized revenue and expenses at the inception of the program (January 1, 2023) based on the model distributed on March 23, 2023. CMS granted overall approval for the eighth round of the program in December 2023. In June 2024, the State of California announced a \$230 million Children's Hospital-Directed Payment Program that will benefit eight California Children's Hospitals. The Hospital is expected to receive an estimated \$53 million of these funds during the next fiscal year.

The Hospital Fee Program is administered pursuant to the Hospital's contract with the Department of Health Care Services (DHCS) and various Medi-Cal-managed care plans to provide health care services to Medi-Cal beneficiaries. The supplemental reimbursement provided in the program is considered in determining the transaction price for services provided to Medi-Cal beneficiaries.

ASC 606, *Revenue from Contracts with Customers*, requires an entity to estimate an amount of variable consideration. The Hospital has concluded that the variable consideration will be determined utilizing the most likely amount method because the Hospital's estimation methodology uses the amounts due and payable based on the model included with the state waiver application, correspondence from the California Hospital Association (CHA), and historical experience with rounds one through four represent the single most likely outcome for the contract. The Hospital, based on its experience, has a reasonable basis to conclude that its estimation methodology best predicts the amount of consideration the Hospital is entitled to receive, including an assessment that it is probable that a significant reversal in the amount of cumulative revenue recognized will not occur when the uncertainty associated with any final retroactive adjustments is subsequently resolved.

The Hospital's primary obligation under the patient service agreements is to deliver health care services for a stipulated fee. The Hospital's performance obligations with respect to the programs are satisfied over time ratably over the program period as defined in the waiver application and California Medicaid state plan amendment.

Accrued Self-Insurance Claims—In the years ended June 30, 2024 and 2023, the Hospital was self-insured for some employee medical health care claims and workers' compensation claims. Self-insurance claims filed and claims incurred but not reported are accrued based upon management's estimates for self-insured claims incurred using actuarial assumptions followed in the insurance industry and the Hospital's historical experience. The Hospital purchases reinsurance policies to insure for both medical health care claims and workers' compensation claims incurred above the self-insured retention limits per claim.

Net Assets without Donor Restrictions—Net assets that are not subject to donor-imposed restrictions and may be expended for any purpose in performing the primary objectives of the

Hospital are classified as net assets without donor restrictions. These net assets may be used at the discretion of the Hospital's management and the Board of Directors.

Net Assets with Donor Restrictions—Net assets with donor restrictions are those whose use by the Hospital has been limited by donors to a specific time period or purpose and are classified as net assets with donor restrictions. This category also includes net assets with donor-imposed restrictions that are to be maintained by the Hospital in perpetuity.

Unconditional promises to give cash and other assets to the Hospital are reported at fair market value at the date the promise is received. Conditional promises to give are reported at fair market value at the date the conditions are satisfied. The gifts are reported as net assets with donor restrictions, if the gifts are received with donor stipulations that limit the use of the donated assets. When a donor restriction expires, that is, when a stipulated time restriction ends or purpose restriction is accomplished, net assets with donor restrictions are reclassified as net assets without donor restrictions and reported in the accompanying consolidated statements of activities as net assets released from restrictions.

Unitrust Agreements—The Hospital has been designated as trustee for several trusts. The trust agreements generally require the Hospital to make payments to beneficiaries based on stipulated interest rates, which range from 4.6% to 15% during the years ended June 30, 2024 and 2023, respectively, applied to the fair market value of the trust assets determined either annually or at the inception of the trust.

Interest Rate Swap—The Hospital accounts for its interest rate swap in accordance with ASC 815, *Derivatives and Hedging*. ASC 815 requires all derivatives to be recorded in the consolidated balance sheets at fair value as either assets or liabilities depending on the rights or obligations under the contract. The Hospital has not designated its derivative financial instrument as a hedge. Accordingly, all unrealized gains and losses are recognized as other gains and losses in the accompanying consolidated statements of activities.

Grants and Contracts Revenues—Grants and contracts revenues generally are recognized as revenues without donor restrictions and grants receivable when the research or educational expenses are incurred or when the contract period has ended, whichever occurs first.

In November 2018, the State of California passed a bond for the benefit of specified children's hospitals, the Children's Hospital Bond Act of 2018, whereby \$1.5 billion was provided to fund grants for construction expansion, renovation, and equipping of qualifying children's hospitals. During the year ended June 30, 2024, funds totaling \$10.6 million were paid to the Hospital, which was included in Proposition 4—building construction advances in the accompanying consolidated statement of changes in net assets. During the year ended June 30, 2023, no funds were paid to the Hospital.

Graduate Medical Education—The Hospital underwrites a large part of the cost of training allied health professionals, physicians, and residents in its emergency rooms, clinics, and inpatient areas. The Hospital recognized revenue of \$15.1 million and \$17.9 million in federal graduate medical education funds during the years ended June 30, 2024 and 2023, respectively, which was included as part of grants, contracts, and other in the consolidated statements of activities.

Fundraising—The Hospital sponsors various philanthropic activities, such as fund-raising events, in addition to maintaining a foundation staff, whose sole purposes are to solicit grants and private

donations and maintain relations with donors and the community. During the years ended June 30, 2024 and 2023, the Hospital incurred \$21.5 million and \$22.7 million, respectively, in fund-raising costs, which were included as part of total expenses before depreciation, amortization, and interest in the consolidated statements of activities.

Functional Expenses—The costs of providing the Hospital’s programs and other activities have been summarized on a functional basis in Note 17. Accordingly, certain costs (primarily information technology and finance) have been allocated among hospital and research and education program services, management and general, and fund-raising support services based on the functional department for which they are incurred. Departmental expenses may include various allocations of costs based on direct assignment, expenses, or other methods.

Income Taxes—The Hospital is recognized by the Internal Revenue Service as exempt from federal income taxes under Section 501(a) of the Internal Revenue Code as charitable organizations qualifying under Section 501(c)(3) and similar California statutes. Income from the Hospital’s limited liability corporations, CHLA ESBT and CHLA Medical Foundation (C corporation), are taxable to CHLA. The income tax provision is recorded in other expenses in the accompanying consolidated financial statements and is immaterial for the years ended June 30, 2024 and 2023.

The Hospital accounts for income taxes in accordance with ASC 740, *Income Taxes*. It is the Hospital’s policy to classify the expense related to interest and penalties, if any, to be paid on underpayments of income taxes within other expenses. The Hospital has evaluated its uncertain tax positions and there were no material penalties or interest recognized for the years ended June 30, 2024 and 2023. The Hospital’s income tax returns for the years ended June 30, 2020, through June 30, 2024, are subject to examination by both the federal and state tax jurisdictions.

Recently Adopted Accounting Pronouncements—In March 2020, the FASB issued ASU No. 2020-04, *Reference Rate Reform (Topic 848): Facilitation of the Effects of Reference Rate Reform on Financial Reporting* (“ASU 2020-04”), which provides companies with temporary optional financial reporting alternatives to ease the potential burden in accounting for reference rate reform and includes a provision that allows companies to account for a modified contract as a continuation of an existing contract. In January 2021, the FASB issued ASU No. 2021-01, *Reference Rate Reform (Topic 848): Scope* (“ASU 2021-01”). ASU 2021-01 clarifies that the practical expedients in ASU 2020-04 apply to derivatives impacted by changes in the interest rate used for margining, discounting, or contract price alignment. The guidance in ASU 2020-04 is optional and may be elected over time, through December 31, 2022, as reference rate reform activities occur. Once ASU 2020-04 is elected, the guidance must be applied prospectively for all eligible contract modifications. In December 2022, the FASB issued ASU No. 2022-06, *Reference Rate Reform (Topic 848): Deferral of the Sunset Date of Topic 848* (“ASU 2022-06”). ASU 2022-06 defers the sunset date of ASU 2020-04 from December 31, 2022, through December 31, 2024. The Hospital has certain debt instruments for which the interest rates are indexed to the London InterBank Offered Rate (LIBOR). To date, the Hospital has amended the interest rate swap agreement to incorporate a fall-back replacement rate indexed to the secured overnight financing rate (SOFR). The Hospital elected to apply the optional expedients, as applicable, to certain modified terms; however, the impact of applying the optional expedients was immaterial to the consolidated financial statements and does not expect the transition to a replacement rate indexed to SOFR to have a material impact on future cash flows. The Hospital will continue to elect to apply the optional expedients to qualifying contract modifications in the future.

In August 2020, the FASB issued ASU No. 2020-06, *Debt—Debt with Conversion and Other Options (Subtopic 470-20) and Derivatives and Hedging—Contracts in Entity’s Own Equity (Subtopic 815-40): Accounting for Convertible Instruments and Contracts in an Entity’s Own Equity* (“ASU 2020-06”), to simplify accounting for certain financial instruments. ASU 2020-06 eliminates the current models that require separation of beneficial conversion and cash conversion features from convertible instruments and simplifies the derivative scope exception guidance pertaining to equity classification of contracts in an entity’s own equity. This new update also introduces additional disclosures for convertible debt and freestanding instruments that are indexed to and settled in an entity’s own equity. This update is effective for fiscal years beginning after December 31, 2023. The Hospital is in the process of evaluating the impact of the new guidance on its consolidated financial statements.

In June 2022, the FASB issued ASU No. 2022-03, *Fair Value Measurement (Topic 820): Fair Value Measurement of Equity Securities Subject to Contractual Sale Restrictions*, which clarifies that contractual sale restrictions are not considered in measuring fair value of equity securities and requires additional disclosures for equity securities subject to contractual sale restrictions. This update is effective for fiscal years beginning after December 15, 2024. The Hospital is in the process of evaluating the impact of this new guidance on its consolidated financial statements.

Subsequent Events—The Hospital has evaluated subsequent events through November 18, 2024, the date the consolidated financial statements were available to be issued. During this period, there were no subsequent events that required recognition or disclosure in the consolidated financial statements.

2. AVAILABILITY AND LIQUIDITY

As of June 30, 2024 and 2023, the table below represents financial assets available for general expenditures (in thousands):

	2024	2023
Financial assets:		
Cash and cash equivalents	\$ 106,006	\$ 41,225
Net patient accounts receivable	309,794	226,106
Contract assets	72,072	58,485
Current portion of pledges receivable—net	35,953	34,421
Investments	<u>640,193</u>	<u>840,843</u>
Total financial assets	1,164,018	1,201,080
Less amounts not available to be used within one year—donor-restricted endowment funds	<u>(361,553)</u>	<u>(315,176)</u>
Financial assets available to meet general expenditures within one year	<u>\$ 802,465</u>	<u>\$ 885,904</u>

As of June 30, 2024 and 2023, the Hospital’s total days cash on hand measured 164 days and 212 days, respectively. Days cash on hand is the number of days that the Hospital can continue to pay its operating expenses, given the amount of cash available. The Hospital Fee Program receivables have been excluded from the table above, due to uncertainty in the timing of collection, which is based on CMS approvals; however, the expectation is that such amounts will be received

within one year. The investments held in board-designated endowment funds (see Note 15) have been included in the table above as certain assets are available for general expenditure within one year in the normal course of operations or board-designated amounts could be made available, if necessary. The Hospital also has a line of credit available to meet short-term needs (see Note 12).

3. NET PATIENT SERVICE REVENUE

The Hospital receives a significant portion of its revenues from Medi-Cal patients and programs related to Medi-Cal (Disproportionate Share Hospital (DSH) and the Hospital Fee Program). As of June 30, 2024 and 2023, the following table shows net patient service revenue by payer (in thousands):

	2024	2023
Medi-Cal	\$ 467,531	\$ 434,426
Hospital provider fee	348,241	336,821
DSH	<u>57,614</u>	<u>43,890</u>
Subtotal	873,386	815,137
Managed care	413,638	348,709
Other	<u>55,262</u>	<u>44,501</u>
Net patient service revenue	<u>\$ 1,342,286</u>	<u>\$ 1,208,347</u>

As of June 30, 2024 and 2023, the following table shows the net accounts receivable by payer:

	2024	2023
Medi-Cal	46 %	43 %
Managed care	42	45
Other	<u>12</u>	<u>12</u>
Total	<u>100 %</u>	<u>100 %</u>

The opening and closing balances of contract assets as of June 30, 2024 and 2023, are as follows (in thousands):

	Contract Asset Balance
June 30, 2023	\$ 58,485
June 30, 2024	<u>72,072</u>
Increase	<u>\$ 13,587</u>

	Contract Asset Balance
June 30, 2022	\$ 44,923
June 30, 2023	<u>58,485</u>
Increase	<u>\$ 13,562</u>

The increase in the contract asset balance as of June 30, 2024, compared to June 30, 2023, is due to the higher average length of stay and acuity level on in-house patients as of the year-end.

Approximately 63% and 81% of the Hospital's contract assets as of June 30, 2024 and 2023, respectively, meet the conditions for unconditional right to payment and are reclassified to patient receivables within 90 days.

The contract asset balance as of July 1, 2023, of \$58.5 million was fully billed during the year ended June 30, 2024, except for \$4.6 million due to unbilled balances related to patients that were in-house at June 30, 2024. The Hospital also recorded additions to unbilled in-house patient accounts of \$67.5 million during the year ended June 30, 2024, resulting in an increase in contract asset balance of \$13.6 million from \$58.5 million to \$72.1 million.

4. CHARITY CARE AND COMMUNITY SERVICES

The Hospital strives to deliver quality health care in a manner that respects the dignity of the individual and family, regardless of ability to pay. Consistent with the Hospital's tax-exempt status and community service responsibilities, the Hospital provides care to patients who meet certain criteria under its charity care policy without charge or at amounts less than its established rates.

Under the Hospital's policy, charity care may be provided to people who are uninsured or underinsured and cannot afford to pay for their own medical care. Since the Hospital does not pursue the collection of amounts determined to qualify as charity care, they are not reported as revenue.

The Hospital provides additional community support through the unpaid cost of public programs, which is defined as the cost of treating Medi-Cal program and indigent beneficiaries in excess of government payments. These costs are calculated through the Hospital's cost accounting system, utilizing standard costing for the services provided and step-down cost allocation methodologies. During the years ended June 30, 2024 and 2023, the unpaid cost of Medi-Cal programs totaled \$306.2 million (unaudited) and \$312.5 million (unaudited), respectively. The Medi-Cal program offsets some of these unpaid costs through the DSH program in the amounts of \$57.6 million and \$43.9 million during the years ended June 30, 2024 and 2023, respectively. In addition, the Hospital Fee Program provided revenue in the amounts of \$348.2 million and \$336.8 million gross of related expenses of \$77.3 million and \$62.6 million in the years ended June 30, 2024 and 2023, respectively (see Note 19).

The Hospital underwrites part of the cost of research that takes place in its facilities. The costs underwritten for research for the years ended June 30, 2024 and 2023, amounted to \$58.0 million (unaudited) and \$64.7 million (unaudited), respectively.

As described in Note 1, the Hospital underwrites a large part of the cost of training allied health professionals, physicians, and residents in its facilities. The costs underwritten for training for the years ended June 30, 2024 and 2023, were \$26.5 million (unaudited) and \$30.7 million (unaudited), respectively.

The Hospital also sponsors various community services to benefit the physically, mentally, and genetically disabled as part of its charitable mission. These services include parental counseling, educational seminars, family support groups, and an outreach organization for families administered by the Hospital in an agency relationship and funded by the State of California. Additionally, a large number of health-related educational programs are provided for the benefit of the community, including health enhancements and wellness, telephone information services, and programs designed to improve the general standards of the health of the community.

5. CONCENTRATIONS OF CREDIT RISK

Financial instruments that potentially subject the Hospital to concentrations of credit risk consist primarily of marketable securities, private investment funds, patient accounts receivable, and pledges receivable. The Hospital's investment portfolios are managed primarily by professional investment managers within the guidelines established by the Board of Directors. These guidelines limit the amounts that may be invested in any one issuer.

Concentration of credit risk with respect to patient accounts receivable from nongovernment payors is limited due to the large number of payors composing the Hospital's patient base.

A significant portion of the Hospital's net patient service revenue is derived from Medi-Cal patients, comprising 65% and 67% of the total, in the years ended June 30, 2024 and 2023, respectively. Excluding the impact of the Hospital Fee Program, revenues from the Medi-Cal program composed of 53% and 55% of total net patient service revenues for the years ended June 30, 2024 and 2023, respectively. The State of California reimburses for inpatient services using a methodology that pays based on diagnosis-related group (DRG). Most inpatient services are reimbursed based upon Medi-Cal All Patient Refined-DRG per-discharge rates. Outpatient services are reimbursed based upon statewide fee schedules.

The California Hospital Fee Program accounted for 26% and 28% of total net patient service revenues in the years ended June 30, 2024 and 2023, respectively. This revenue is included in total Medi-Cal patient revenue discussed in the paragraph above.

The State of California provides supplemental Medi-Cal payments to hospitals that serve a disproportionately high percentage of Medi-Cal and other low-income patients through DSH. As discussed in Note 4, the Hospital has recorded \$57.6 million and \$43.9 million in the years ended June 30, 2024 and 2023, respectively, in net patient service revenue relating to DSH.

Certain government payors provide for payments to the Hospital at tentative rates, with final settlement determined after submission of annual reports by the Hospital and audits thereof by the payors. As of June 30, 2024 and 2023, settlement amounts receivable of \$41.3 million and \$29.4 million, respectively, are included in receivables under government and state programs, settlement amounts receivable of \$29 million and \$0 million, respectively, are included in receivables under other assets, and payables, including estimates resulting from Medi-Cal cost report audits, of \$12.2 million and \$19.6 million, respectively, are included in payables under government programs and \$14.9 million and \$32.3 million, respectively, within other noncurrent

liabilities in the accompanying consolidated balance sheets. Changes in estimates recorded as a result of audit settlements for prior periods increased net patient service revenue by \$11.7 million and increased net patient service revenue by \$4.0 million in the years ended June 30, 2024 and 2023, respectively.

6. INVESTMENTS

The carrying amounts of investments as of June 30, 2024 and 2023, are as follows (in thousands):

	2024	2023
Current portion of trustee-held funds (Note 10)	\$ 7,631	\$ 7,503
Cash equivalents and short-term investments	18,274	3,433
Mutual funds	284,793	388,772
Equity securities	13,928	19,899
Debt securities	243,685	322,653
Private investment funds	82,047	108,700
Commingled funds	3,885	3,437
Assets limited as to use—net of current portion	646,612	846,894
Total investments	\$ 654,243	\$ 854,397

The cash and cash equivalents included in investments at June 30, 2024 and 2023, represent cash held by the Hospital's investment managers that will be invested based on the Hospital's long-term investment strategy.

Investment income and gains and losses for assets limited as to use, cash equivalents, and other investments for the years ended June 30, 2024 and 2023, are composed of the following (in thousands):

	2024	2023
Investment income—net of fees	\$ 6,648	\$ 12,978
Realized gain on sale of investments—net	18,077	2,206
Unrealized gain on investments—net	9,521	21,681
Without donor restriction net investment gain	34,246	36,865
Investment income—net of fees	9,367	16,239
Realized gain on sale of investments—net	23,028	2,625
Unrealized gain on investments—net	9,552	23,794
With donor restriction net investment gain	41,947	42,658
Total	\$ 76,193	\$ 79,523

7. FAIR VALUE MEASUREMENTS

The Hospital reports fair value measurements in accordance with ASC 820, *Fair Value Measurements and Disclosures*, which defines fair value, establishes a framework for measuring fair value in accordance with existing US GAAP, and provides expanded disclosures about fair value

measurements. Assets and liabilities recorded at fair value in the consolidated balance sheets are categorized based upon the level of judgment associated with the inputs used to measure their fair value and the level of market price observability.

Due to the short-term nature of cash and cash equivalents; patient accounts receivable; Hospital Fee Program balances; receivables and payables under government and state programs; accounts payable and accrued expenses; and salaries, wages, and related liabilities, their carrying values approximate their fair values.

Pledges receivable are discounted at the current market interest rate at the time of receipt, which approximates their fair value.

The fair values of liabilities under unitrust agreements are determined by utilizing discount rates commensurate with the corresponding investments at the time of receipt, which approximate their fair value.

Long-term debt is carried at amortized cost in the consolidated financial statements; however, in accordance with ASC 820, the Hospital is required to disclose the fair value. The fair values of the Hospital's long-term debt, including the 2017 and 2022 bonds, are based on current trading values at or near the last business day of the fiscal year. At June 30, 2024, the fair value was \$421.0 million compared to the carrying value of \$432.2 million. At June 30, 2023, the fair value was \$430.0 million compared to the carrying value of \$442.9 million.

Investments measured and reported at fair value using level inputs, as defined by ASC 820, are classified and disclosed in one of the following categories:

Level 1—Quoted prices in active markets for identical investments available as of the reporting date.

Level 2—Pricing inputs are other-than-quoted prices in active markets, which are either directly or indirectly observable as of the reporting date, and fair value is determined through the use of models or other valuation methodologies.

Level 3—Pricing inputs that are generally unobservable and typically reflect management's estimates of assumptions that market participants would use in pricing the asset or liability.

In certain cases, the inputs used to measure fair value may fall into different levels of the fair value hierarchy. In such cases, an investment's level within the fair value hierarchy is based on the lowest level of input that is significant to the fair value measurement.

The following is a description of valuation inputs and techniques that the Hospital utilizes to determine the fair value of each major category of assets and liabilities in accordance with ASC 820:

Mutual Funds—Registered with the Securities and Exchange Commission as mutual funds under the Investment Company Act of 1940. To the extent valuation adjustments are not applied, mutual funds are categorized as Level 1.

Equity Securities (US and Foreign)—Equity securities that are actively traded on a securities exchange are valued based on quoted prices from the applicable exchange, and to the extent valuation adjustments are not applied to these securities, they are categorized as Level 1. Equity securities traded on inactive markets and valued using other observable inputs are categorized as Level 2.

Debt Securities (Corporate Bonds)—Investment-grade bonds are valued using inputs and techniques, which include third-party pricing vendors, dealer quotations, and recently executed transactions in securities of the issuer or comparable issuers. Adjustments to individual bonds can be applied to recognize trading differences compared to other bonds issued by the same issuer. Values for high-yield bonds are based primarily on pricing vendors and dealer quotations from relevant market makers. The dealer quotations received are supported by credit analysis of the issuer that takes into consideration credit-quality assessments; daily trading activity; and the activity of the underlying equities, listed bonds, and sector-specific trends. To the extent that these inputs are observable and timely, the values of corporate bonds are categorized as Level 2.

Debt Securities (US Treasury Notes and Trustee-Held Funds)—US Treasury notes are valued based on prices provided by third-party vendors that obtain feeds from a number of live data sources, including active market makers and interdealer brokers. To the extent that the values are actively quoted, they are categorized as Level 1.

Investment Funds (Private Investment Funds and Commingled Funds)—Investment funds are valued based upon the NAV per share (or its equivalent) of the underlying fund as provided by the investment manager. Such funds are excluded from the fair value hierarchy-level categories.

Derivative Instruments—The fair values of interest rate swaps are estimated using various inputs, including quotations from various dealers and counterparties, and pricing models that use certain observable inputs, such as the creditworthiness of the counterparties, default probabilities, yield curves, and credit curves. The pricing models utilized generally do not entail material subjectivity because the methodologies employed do not necessitate significant judgments. If the pricing inputs are observed from actively quoted markets, the derivative values are categorized as Level 2. Interest rate swaps are valued in accordance with the terms of each contract based on current interest rate spreads. Market standard pricing models are used for valuing interest rate swaps.

The following tables present information about the Hospital's invested assets and related liabilities measured at fair value on a recurring basis as of June 30, 2024 and 2023, and indicate the fair value hierarchy of the valuation techniques utilized by management to determine such fair value (in thousands):

Assets	2024					
	Investments at Fair Value			Fair Value	Investments at NAV	Total
	Level 1	Level 2	Level 3			
Investments:						
Cash equivalents and short-term investments	\$ 18,274	\$ -	\$ -	\$ 18,274	\$ -	\$ 18,274
Mutual funds:						
Global (US and foreign) equity funds	229,818	-	-	229,818	-	229,818
Fixed-income funds	54,975	-	-	54,975	-	54,975
Equity securities:						
US equity securities	13,823	105	-	13,928	-	13,928
Debt securities:						
Corporate bonds	-	131,290	-	131,290	-	131,290
Government bonds	33,118	79,277	-	112,395	-	112,395
Investments measured at NAV:						
Private investment funds:						
Foreign equity funds	-	-	-	-	78,036	78,036
Hedge funds—alternative investments	-	-	-	-	7	7
Private equity funds	-	-	-	-	3,459	3,459
Venture capital funds	-	-	-	-	545	545
Commingled funds:						
Common stock	-	-	-	-	3,245	3,245
Corporate bonds	-	-	-	-	640	640
Total assets limited as to use	350,008	210,672	-	560,680	85,932	646,612
Trustee-held funds	7,631	-	-	7,631	-	7,631
Total investments	<u>\$357,639</u>	<u>\$210,672</u>	<u>\$ -</u>	<u>\$568,311</u>	<u>\$85,932</u>	<u>\$654,243</u>
Liabilities						
Interest rate swap liability	<u>\$ -</u>	<u>\$ (2,774)</u>	<u>\$ -</u>	<u>\$ (2,774)</u>	<u>\$ -</u>	<u>\$ (2,774)</u>

Assets	2023					
	Investments at Fair Value			Fair Value	Investments at NAV	Total
	Level 1	Level 2	Level 3			
Investments:						
Cash equivalents and short-term investments	\$ 3,433	\$ -	\$ -	\$ 3,433	\$ -	\$ 3,433
Mutual funds:						
Global (US and foreign) equity funds	315,099	-	-	315,099	-	315,099
Fixed-income funds	73,673	-	-	73,673	-	73,673
Equity securities:						
US equity securities	19,711	188	-	19,899	-	19,899
Debt securities:						
Corporate bonds	646	208,161	-	208,807	-	208,807
Government bonds	113,846	-	-	113,846	-	113,846
Investments measured at NAV:						
Private investment funds:						
Foreign equity funds	-	-	-	-	105,167	105,167
Hedge funds—alternative investments	-	-	-	-	10	10
Private equity funds	-	-	-	-	2,899	2,899
Venture capital funds	-	-	-	-	624	624
Commingled funds:						
Common stock	-	-	-	-	2,568	2,568
Corporate bonds	-	-	-	-	869	869
Total assets limited as to use	526,408	208,349	-	734,757	112,137	846,894
Trustee-held funds	7,503	-	-	7,503	-	7,503
Total investments	<u>\$533,911</u>	<u>\$208,349</u>	<u>\$ -</u>	<u>\$742,260</u>	<u>\$112,137</u>	<u>\$854,397</u>
Liabilities						
Interest rate swap liability	<u>\$ -</u>	<u>\$ (3,896)</u>	<u>\$ -</u>	<u>\$ (3,896)</u>	<u>\$ -</u>	<u>\$ (3,896)</u>

Transfers in or out are recognized based on the beginning fair value of the year in which they occurred. There were no significant transfers on investments between Level 1 and Level 2 during the years ended June 30, 2024 and 2023. There were no transfers to or transfers from Level 3 investments during the years ended June 30, 2024 and 2023.

The Hospital classified \$85.9 million and \$112.1 million of investments measured at the NAV per share (or its equivalent), for which the practical expedient was applied, as of June 30, 2024 and 2023, respectively (see Note 1). As of June 30, 2024 and 2023, substantially all of the investments measured at NAV can be redeemed daily and have no redemption restrictions or unfunded capital commitments.

8. PLEDGES RECEIVABLE

Outstanding pledges and contributions receivable from various corporations, foundations, and individuals as of June 30, 2024 and 2023, are reported net of an estimated allowance for uncollectible pledges and are as follows (in thousands):

	2024	2023
Pledges due:		
In less than one year	\$ 40,633	\$ 37,925
In one to five years	25,052	27,364
After five years	2,200	3,000
Discount on pledges	(2,309)	(2,104)
Allowance for uncollectible pledges	<u>(5,518)</u>	<u>(4,478)</u>
Total pledges receivable—net	<u>\$ 60,058</u>	<u>\$ 61,707</u>

9. TRUSTEE-HELD FUNDS

As of June 30, 2024 and 2023, the current portion of trustee-held funds amounted to \$7.6 million and \$7.5 million, respectively, related to interest due on the Series 2017A and Series 2022 Bonds and interest income earned on the 2017A Project Fund, which can only be used to redeem the bonds in accordance with the agreement.

10. PROPERTY, PLANT, AND EQUIPMENT

A summary of property, plant, and equipment as of June 30, 2024 and 2023, is as follows (in thousands):

	2024	2023
Buildings and improvements	\$ 1,176,193	\$ 1,157,818
Equipment	723,951	677,879
Accumulated depreciation	<u>(1,166,699)</u>	<u>(1,089,003)</u>
Property, plant, and equipment—net	733,445	746,694
Construction in progress	116,668	119,661
Land	<u>138,422</u>	<u>138,422</u>
Total property, plant, and equipment—net	<u>\$ 988,535</u>	<u>\$ 1,004,777</u>

Depreciation expenses for the years ended June 30, 2024 and 2023, amounted to \$77.8 million and \$69.4 million, respectively.

11. LIABILITIES TO THIRD-PARTY PAYORS

In February 2024, the Hospital learned that a cyber security threat actor had gained access to some of the information technology systems of Change Healthcare, a subsidiary of UnitedHealth Group (UHG), representing a vendor related to the Hospital's claims processing activity. UHG isolated the affected systems upon learning of this threat and Change Healthcare suspended its claims processing activity. During the year ended June 30, 2024, the Hospital participated in UHG's Optum Temporary Funding Assistance Program, which was designed to provide short-term cash flow relief to providers affected by the disruption in Change Healthcare's services. As of June 30, 2024, the Hospital received \$55.3 million under this program, which is expected to be repaid 45 days from the date of invoice, as determined and submitted by Change Healthcare, and accordingly the balance is included in current liabilities in the accompanying consolidated balance sheets. The Hospital has not incurred any fees, interest, or other associated costs for participating in the program.

12. LINES OF CREDIT

On May 29, 2020, the Hospital entered into a \$50 million revolving line of credit with a bank, secured under the Hospital's existing Master Trust Indenture (see Note 13). Borrowings under the line bear interest at the Bloomberg Short-Term Bank Yield rate, plus 1.25%. Accrued interest is due monthly with principal due at maturity (May 25, 2024). The line of credit was extended upon maturity until May 23, 2025. There was an outstanding balance of \$50 million at June 30, 2024 and 2023, respectively, which are included in current liabilities in the accompanying consolidated balance sheets.

On August 4, 2023, the Hospital entered into a \$30 million revolving line of credit with a bank, secured by interest in certain investment funds. Borrowings under the line bear interest at SOFR, plus 1.10%. Accrued interest is due monthly with principal due at maturity (August 4, 2024). On April 11, 2024, the line of credit was modified and amended to reflect the principal amount of \$50 million. Borrowings under the line bear interest at SOFR, plus 1.10%. Accrued interest is due monthly with principal due upon demand, with a final maturity date of August 2, 2025. No amounts were outstanding as of June 30, 2024.

13. LONG-TERM DEBT

Long-term debt as of June 30, 2024 and 2023, consists of the following obligations (in thousands):

	2024	2023
Revenue Bond Series 2022, consisting of Series 2022A-1 Revenue Bonds issued for \$51,720, with serial maturities through 2034, interest payable semiannually at various fixed rates, and Series 2022A-2 Revenue Bonds issued for \$51,700, with serial maturities through 2034, and interest payable monthly at various fixed rates, both series collateralized by a pledge of gross revenues and a mortgage of the Hospital's real and personal assets, and other cash and investments.	\$ 93,470	\$103,420
Revenue Bond Series 2017, consisting of Series 2017A Revenue Bonds issued for \$274,520, with serial maturities between 2030 and 2049, interest payable semiannually at various fixed rates, and Series 2017B Variable Rate Revenue Bonds issued for \$52,180, with serial maturities from 2049 to 2051, and interest payable monthly, both series collateralized by a pledge of gross revenues and a mortgage of the Hospital's real and personal assets, and other cash and investments—net of unamortized premium of \$14,747 and \$15,696 at June 30, 2024 and 2023, respectively.	<u>341,447</u>	<u>342,396</u>
Total revenue bonds	434,917	445,816
Less portion classified as current	(10,250)	(9,950)
Net of bond issuance cost	<u>(2,672)</u>	<u>(2,964)</u>
Total long-term debt obligations	<u>\$421,995</u>	<u>\$432,902</u>

On August 31, 2022, the Hospital issued California Health Facilities Financing Authority Revenue Bonds (CHLA) for \$103.4 million, consisting of Series 2022A-1 Revenue Bonds (the "Series 2022A-1 Bonds") for \$51.7 million and Series 2022A-2 Revenue Bonds (the "Series 2022A-2 Bonds") for \$51.7 million. Financing costs are amortized over the life of the bonds. The Series 2022A-1 Bonds bear a fixed interest rate of 3.12%, with interest payable semiannually, and have serial maturity dates beginning November 2023 through 2034. The Series 2022A-2 Bonds bear a fixed interest rate of 3.08%, with interest payable monthly, and have serial maturity dates beginning November 2023 through 2034.

The proceeds from the Series 2022A-1 Bonds and Series 2022A-2 Bonds were used to refund the outstanding principal amount of the Series 2012A Bonds.

On June 6, 2017, the Hospital issued California Health Facilities Financing Authority Revenue Bonds (CHLA) for \$326.7 million, consisting of Series 2017A Revenue Bonds (the "Series 2017A Bonds") for \$274.5 million and Series 2017B Variable Rate Revenue Bonds (the "Series 2017B Bonds") for \$52.2 million. These bonds were issued at a premium of \$21.5 million. Financing costs are amortized over the life of the bonds. The Series 2017A Bonds bear fixed interest rates ranging from 4% to 5%, with interest payable semiannually, and have serial maturity dates beginning in August 2030 and through 2049. The Series 2017B Bonds, which mature in 2051, bear interest at a Bank-Bought index rate, with a spread of 0.99%, consisting of a LIBOR index rate, for a Bank-Bought index rate period consisting of a LIBOR index rate period, commencing on June 6, 2017, and ending on June 5, 2024. The bonds are subject to mandatory tender for purchase on June 6, 2024. On May 15, 2024, the Hospital extended the mandatory purchase date applicable to the bonds from June 6,

2024, to June 1, 2029. In June 2018, Global Regulators announced that the LIBOR Index would be replaced; subsequently, it was announced that LIBOR would be discontinued at the end of 2023. On June 21, 2023, the Hospital modified the variable rate from LIBOR to SOFR. The change is consistent with the LIBOR Fallback Protocols and was documented through an Amended and Restate Indenture. Interest is payable monthly, with weekly rates ranging from 4.63% to 5.03%, annualized, for the year ended June 30, 2024, and 2.19% to 4.61%, annualized, for the year ended June 30, 2023. The proceeds from the Series 2017A Bonds and Series 2017B Bonds were used to (i) refund the outstanding principal amount of the Series 2012B Variable Rate Revenue Bonds (the "Series 2012B Bonds"), the outstanding principal amount of the Series 2010A Bonds, and the outstanding principal amount of the Series 2007 Bonds; (ii) finance the project (includes the purchase of equipment, the upgrade of the Hospital's information system, and the interior renovations of various facilities); and (iii) pay certain of the costs of issuance on the bonds.

The swap agreement is currently used to hedge the variable interest rate portion of the Series 2017B Bonds. This interest rate swap has a notional amount of \$43.3 million and matures on February 15, 2034. The agreement effectively adjusts the Hospital's interest costs by swapping the variable interest rate on the Series 2017B Bonds to a fixed rate of 3.626%. The interest rate swap agreement had a fair value of \$(2.8) million and \$(3.9) million at June 30, 2024 and 2023, respectively, which represents the estimated amount the Hospital would pay to terminate the swap via a competitively priced assignment. Because the swap is not designated as a cash flow hedge in accordance with ASC 815, the change in fair value is reported as interest rate swap mark-to-market gain in the accompanying consolidated statements of activities. As of June 30, 2024 and 2023, the Hospital has posted \$0 million and \$0 million, respectively, of collateral related to the swap agreement, which is included in other noncurrent assets in the accompanying consolidated balance sheets.

Pursuant to the Master Indenture, the Hospital has pledged its gross revenues as security for all of its long-term debt. The Master Indenture provides for limitations on the incurrence of additional indebtedness, unless certain conditions are met, and requires the Hospital to maintain an annual long-term debt service coverage ratio of at least 1.10x. The Hospital complied with all covenant

requirements pursuant to the Master Indenture at June 30, 2024 and 2023. The aggregate principal maturities and sinking fund requirements of long-term debt as of June 30, 2024, are as follows (in thousands):

**Years Ending
June 30**

2025	\$ 10,250
2026	10,575
2027	11,240
2028	11,695
2029	12,050
Thereafter	<u>364,360</u>
Total principal maturities	420,170
Net unamortized premium	14,747
Unamortized bond issuance costs	<u>(2,672)</u>
Total	<u>\$ 432,245</u>

The premium on the 2017 bonds is being amortized over the life of the bonds using the effective interest method. Net discount and premium amortization during the years ended June 30, 2024 and 2023, amounted to \$0.9 million and \$1.1 million, respectively.

Cash paid for interest during the years ended June 30, 2024 and 2023, amounted to \$20.5 million and \$19.2 million, respectively.

14. COMMITMENTS AND CONTINGENCIES

Since 1994, the Hospital has been insured on a claims-made basis for both professional/general liability coverage and directors' and officers' coverage with per claim deductible amounts of \$250 thousand and \$25 thousand, respectively. An estimated liability of claims, incurred but not reported, of \$4.3 million and \$3.8 million as of June 30, 2024 and 2023, respectively, has been accrued and is included in other noncurrent liabilities in the accompanying consolidated balance sheets.

The Hospital is self-insured for workers' compensation benefits for employees. Annual expenses and funding under this program are based on past claims experience and projected losses. Insurance coverage, in excess of the per occurrence, self-insured retention, has been secured with insurers. Actuarial estimates of uninsured losses for the workers' compensation program are \$15.1 million and \$15.4 million at June 30, 2024 and 2023, respectively, and include an estimate for claims incurred but not reported. These amounts are included in other noncurrent liabilities in the accompanying consolidated balance sheets, net of the current portions of \$5.0 million and \$4.3 million as of June 30, 2024 and 2023, respectively, which are included in salaries, wages, and related liabilities. These amounts are gross of estimated reinsurance recoverable, totaling \$434 thousand and \$502 thousand as of June 30, 2024 and 2023, respectively, which are included in other assets in the accompanying consolidated balance sheets.

The Hospital self-insured a portion of its employees for group medical benefits. Actuarial estimates of claims, incurred but not reported, for the group insurance program were \$6.7 million and

\$6.6 million as of June 30, 2024 and 2023, respectively, which are included in salaries, wages, and related liabilities. These amounts are the gross of estimated reinsurance recoverable, totaling \$1.3 million and \$871 thousand as of June 30, 2024 and 2023, respectively, which are included in other current assets in the accompanying consolidated balance sheets.

From time to time, the Hospital is subject to legal proceedings, claims, and litigation arising in the ordinary course of business. The Hospital defends itself vigorously against any such claims. Although the outcome of these matters is currently not determinable, management expects that any losses that are probable or have a reasonable possibility of being incurred in excess of amounts already accrued in its consolidated balance sheets would not be material to the consolidated financial statements as a whole.

15. NET ASSETS WITH DONOR RESTRICTIONS

Net assets with donor restrictions as of June 30, 2024 and 2023, are available for the following purposes (in thousands):

	2024	2023
Hospital programs	\$ 503,323	\$ 502,530
Capital projects	29,515	34,634
Other programs	<u>60,058</u>	<u>61,708</u>
Total net assets with donor restrictions	<u>\$ 592,896</u>	<u>\$ 598,872</u>

The investment earnings from net assets with donor restrictions are restricted for Hospital clinical research and academic programs, as well as capital projects, while the corpus may not be expended.

As of June 30, 2024, the Hospital's endowment consisted of 353 individual funds established for various purposes. The total endowment includes both donor-restricted endowment funds and funds designated by the Board of Directors to function as endowments. As required by US GAAP, net assets associated with endowment funds, including funds designated by the Board of Directors to function as endowments, are classified and reported based on the existence or absence of donor-imposed restrictions.

The Hospital is subject to the State Prudent Management of Institutional Funds Act (SPMIFA) and, thus, classifies amounts in its donor-restricted endowment funds as net assets with donor restrictions because those net assets are time-restricted until the Board of Directors appropriates such amounts for expenditure. Most of those net assets are also subject to purpose restrictions that must be met before reclassifying those net assets to net assets without donor restrictions. The Board of Directors has interpreted SPMIFA as not requiring the maintenance of purchasing power of the original gift amount contributed to an endowment fund, unless a donor stipulates the contrary. As a result of this interpretation, when reviewing its donor-restricted endowment funds, the Hospital considers a fund to be underwater if the fair value of the fund is less than the sum of (a) the original value of initial and subsequent gift amounts donated to the fund and (b) any accumulations to the fund that are required to be maintained in perpetuity in accordance with the direction of the applicable donor gift instrument. The Hospital has interpreted SPMIFA to permit spending from underwater funds in accordance with the prudent measures required under the law.

Additionally, in accordance with SPMIFA, the Hospital considers the following factors in making a determination to appropriate or accumulate donor-restricted endowment funds:

The duration and preservation of the fund

The purposes of the Hospital and the donor-restricted endowment fund

General economic conditions

The possible effect of inflation and deflation

The expected total return from income and the appreciation of investments

Other resources of the Hospital

The investment policies of the Hospital

The endowment net assets composition by type of fund as of June 30, 2024 and 2023, is as follows (in thousands):

	Without Donor Restrictions	With Donor Restrictions	Total
2024			
Donor-restricted endowment funds	\$ 192	\$ 361,361	\$ 361,553
Board-designated endowment funds	<u>411,234</u>	<u>-</u>	<u>411,234</u>
Total funds	<u>\$ 411,426</u>	<u>\$ 361,361</u>	<u>\$ 772,787</u>
2023			
Donor-restricted endowment funds	\$ 1,350	\$ 313,826	\$ 315,176
Board-designated endowment funds	<u>401,202</u>	<u>-</u>	<u>401,202</u>
Total funds	<u>\$ 402,552</u>	<u>\$ 313,826</u>	<u>\$ 716,378</u>

The changes in endowment net assets for the years ended June 30, 2024 and 2023, are as follows (in thousands):

	Without Donor Restrictions	With Donor Restrictions	Total
Endowment net assets—June 30, 2022	<u>\$ 379,985</u>	<u>\$ 284,237</u>	<u>\$ 664,222</u>
Investment return:			
Investment income and realized gain	15,184	9,064	24,248
Unrealized loss	<u>21,681</u>	<u>11,331</u>	<u>33,012</u>
Total investment return	<u>36,865</u>	<u>20,395</u>	<u>57,260</u>
Contributions	-	22,653	22,653
Transfer to board-designated endowment funds	80,510	-	80,510
Amounts appropriated for expenditure	(94,808)	(10,745)	(105,553)
Other changes—net	<u>-</u>	<u>(2,714)</u>	<u>(2,714)</u>
Endowment net assets—June 30, 2023	<u>402,552</u>	<u>313,826</u>	<u>716,378</u>
Investment return:			
Investment income and realized gain	24,725	16,682	41,407
Unrealized gain	<u>9,521</u>	<u>5,267</u>	<u>14,788</u>
Total investment return	<u>34,246</u>	<u>21,949</u>	<u>56,195</u>
Contributions	-	34,475	34,475
Transfer to board-designated endowment funds	47,249	-	47,249
Amounts appropriated for expenditure	(72,621)	(10,348)	(82,969)
Other changes—net	<u>-</u>	<u>1,459</u>	<u>1,459</u>
Endowment net assets—June 30, 2024	<u><u>\$ 411,426</u></u>	<u><u>\$ 361,361</u></u>	<u><u>\$ 772,787</u></u>

The description of the amounts classified as net assets with donor restrictions as of June 30, 2024 and 2023, is as follows (in thousands):

	2024	2023
Net assets with donor restrictions—the portion of perpetual endowment funds that is required to be retained permanently either by explicit donor stipulation or by SPMIFA:		
With purpose restrictions—not related to capital acquisitions	\$ 281,293	\$ 246,403
With purpose restrictions—for capital acquisitions	3,587	3,587
Without purpose restrictions	<u>29,124</u>	<u>29,124</u>
Total endowment funds classified as net assets with donor restrictions net assets	<u>\$ 314,004</u>	<u>\$ 279,114</u>
Net assets with donor restrictions—the portion of perpetual endowment funds under SPMIFA—with purpose restrictions	<u>\$ 47,357</u>	<u>\$ 34,711</u>

From time to time, the fair value of assets associated with individual donor-restricted endowment funds may fall below the level that the donor or SPMIFA requires the Hospital to retain as a fund of perpetual duration. Deficiencies of this nature are reported in net assets without donor restrictions. As of June 30, 2024, certain endowment funds had a fair value less than the original gift value, which had an original gift value of \$3.0 million, a current fair value of \$2.9 million, and a deficiency of \$75 thousand. As of June 30, 2023, certain endowment funds had a fair value less than the original gift value, which had an original gift value of \$6.7 million, a current fair value of \$6.4 million, and a deficiency of \$267 thousand. These deficiencies resulted from unfavorable market fluctuations and/or continued appropriations for certain programs that were deemed prudent by the Hospital.

The Hospital has adopted investment and spending policies for endowment assets that attempt to provide a predictable stream of funding to programs supported by its endowment while seeking to maintain the purchasing power of the endowment assets. Endowment assets include those assets of donor-restricted funds that the organization must hold in perpetuity or for donor-specified periods. Under this policy, as approved by the Board of Directors, the endowment assets are invested in a manner that is intended to produce a real rate of return of greater than 5.5% (net of fees and adjusted for inflation), as calculated based on rolling five-year periods. Actual returns in any given year may vary from this amount.

To satisfy its long-term rate-of-return objectives, the Hospital relies on a total return strategy in which investment returns are achieved through both capital appreciation (realized and unrealized) and current yield (interest and dividends). The Hospital targets a diversified asset allocation intended to achieve its long-term return objectives within prudent risk constraints.

The Hospital has a policy of appropriating for distribution each year a portion of its endowment fund's average fair value over the prior 12 quarters as of the calendar year-end preceding the fiscal year in which the distribution is planned. In establishing its policy, the Hospital considered the long-term expected return on its endowment. The rate of appropriation for distribution was 4.25% for the years ended June 30, 2024 and 2023. The Hospital permits spending from underwater

endowment funds depending on the degree to which the fund is underwater, unless otherwise precluded by donor stipulations or relevant laws and regulations.

16. EMPLOYEE BENEFIT PLANS

The Hospital maintains a defined contribution plan for the benefit of its employees. All employees are eligible to defer a percentage of their salaries up to the maximum amount allowed by the Internal Revenue Service. Employees who have satisfied minimum service requirements are eligible to receive an employer contribution from the Hospital. The Hospital matches 100% of employee contributions up to the first 3% of employee pay for participating employees. The Hospital's contribution expense was \$15.2 million and \$13.5 million for the years ended June 30, 2024 and 2023, respectively.

17. FUNCTIONAL EXPENSES

Functional expenses for the years ended June 30, 2024 and 2023, are as follows (in thousands):

	<u>Program Services</u>		<u>Support Services</u>		2024
	Hospital	Research and Education	Management and General	Fundraising	Total Expenses
Salaries and employee benefits	\$ 672,634	\$ 96,766	\$186,293	\$14,401	\$ 970,094
Professional fees and purchased services	153,679	61,404	94,110	4,781	313,974
Supplies	250,130	5,566	10,037	192	265,925
Hospital Fee Program	77,274	-	-	-	77,274
Depreciation and amortization	71,856	2,761	3,446	-	78,063
Interest	20,438	-	884	-	21,322
Other	<u>50,353</u>	<u>9,715</u>	<u>29,525</u>	<u>2,167</u>	<u>91,760</u>
Total	<u>\$1,296,364</u>	<u>\$176,212</u>	<u>\$324,295</u>	<u>\$21,541</u>	<u>\$1,818,412</u>
	<u>Program Services</u>		<u>Support Services</u>		2023
	Hospital	Research and Education	Management and General	Fundraising	Total Expenses
Salaries and employee benefits	\$ 594,693	\$ 87,889	\$184,164	\$13,118	\$ 879,864
Professional fees and purchased services	143,359	56,858	98,292	6,418	304,927
Supplies	205,687	5,518	12,424	170	223,799
Hospital Fee Program	62,631	-	-	-	62,631
Depreciation and amortization	64,392	2,231	3,057	-	69,680
Interest	18,809	-	876	-	19,685
Other	<u>49,188</u>	<u>8,872</u>	<u>31,292</u>	<u>3,008</u>	<u>92,360</u>
Total	<u>\$1,138,759</u>	<u>\$161,368</u>	<u>\$330,105</u>	<u>\$22,714</u>	<u>\$1,652,946</u>

18. RELATED-PARTY TRANSACTIONS

Pediatric Management Group (PMG) is a California limited liability company that manages the practice of Children's Hospital Los Angeles Medical Group (CHLAMG) based at the Hospital. The Hospital has recorded payables to PMG and CHLAMG in the amount of \$12.2 million and \$4.8 million at June 30, 2024 and 2023, respectively. These payables relate to expenses incurred by the Hospital on behalf of PMG and/or CHLAMG.

19. HOSPITAL FEE PROGRAM

In October 2010, CMS substantially approved the Hospital Fee Program, and the State of California began its implementation. The supplemental payments encompass fee-for-service payments directly from the DHCS, as well as payments routed through managed care plans.

To date, there have been eight segments (or “rounds”) of the Hospital Fee Program, covering the service period from April 2009 through December 2024 in segments of varying duration. Each round of legislation is subject to CMS review and approval. The Hospital incurs two types of expenses related to the Hospital Fee Program, the largest of which is the QAF, which is paid directly to the State of California and then matched by federal funds. A pledge payment is made to the California Health Foundation and Trust (CHFT). The CHFT is a NFP entity created by CHA for several purposes, one of which is to aggregate and distribute financial resources to support charitable activities at various hospitals and health systems in California. Hospital Fee Program pledge payments received by the CHFT are aggregated from all participating hospitals and used to “make whole” those participants in the program who have made QAF payments to the state in excess of fee-for-service program revenue returned. The Hospital recognizes QAF and CHFT pledge expenses upon approval of the related coverage period by CMS.

Significant timing differences have occurred between revenue recognition in the consolidated financial statements and related cash transactions. The Hospital recorded \$284.0 million in receivables and \$88.4 million in payables as of June 30, 2024, related to this program. The Hospital recorded \$294 million in receivables and \$68.4 million in payables as of June 30, 2023, related to this program.

The following table shows financial activity related to the Hospital Fee Program both as recognized in the consolidated statements of activities and based on cash receipts and disbursements (in thousands):

Statement of Activities		Fee for Service	Managed Care	Total Revenues	QAF	CHFT Pledge	Total Expenses	Net Revenue/ Proceeds
FY 2023	Round 5	\$ -	\$ -	\$ -	\$ -	\$ -	\$ -	\$ -
	Round 6	-	11,815	11,815	-	-	-	11,815
	Round 7	101,498	52,660	154,158	28,488	1,564	30,052	124,106
	Round 8	<u>101,658</u>	<u>69,190</u>	<u>170,848</u>	<u>31,829</u>	<u>750</u>	<u>32,579</u>	<u>138,269</u>
		<u>\$203,156</u>	<u>\$133,665</u>	<u>\$336,821</u>	<u>\$60,317</u>	<u>\$ 2,314</u>	<u>\$62,631</u>	<u>\$274,190</u>
FY 2024	Round 4	\$ -	\$ -	\$ -	\$ -	\$(1,820)	\$(1,820)	\$ 1,820
	Round 5	-	-	-	-	-	-	-
	Round 6	-	-	-	-	-	-	-
	Round 7	-	(1,348)	(1,348)	-	-	-	(1,348)
	Round 8	<u>200,569</u>	<u>149,020</u>	<u>349,589</u>	<u>70,408</u>	<u>8,686</u>	<u>79,094</u>	<u>270,495</u>
		<u>\$200,569</u>	<u>\$147,672</u>	<u>\$348,241</u>	<u>\$70,408</u>	<u>\$ 6,866</u>	<u>\$77,274</u>	<u>\$270,967</u>
Cash Activities								
FY 2023	Round 5	\$ -	\$ -	\$ -	\$ -	\$ -	\$ -	\$ -
	Round 6	-	29,472	29,472	13,315	321	13,636	15,836
	Round 7	182,870	-	182,870	29,868	2,994	32,862	150,008
	Round 8	<u>-</u>	<u>-</u>	<u>-</u>	<u>-</u>	<u>-</u>	<u>-</u>	<u>-</u>
		<u>\$182,870</u>	<u>\$ 29,472</u>	<u>\$212,342</u>	<u>\$43,183</u>	<u>\$ 3,315</u>	<u>\$46,498</u>	<u>\$165,844</u>
FY 2024	Round 5	\$ -	\$ -	\$ -	\$ -	\$ -	\$ -	\$ -
	Round 6	-	15,062	15,062	7,508	158	7,666	7,396
	Round 7	-	88,014	88,014	16,027	563	16,590	71,424
	Round 8	<u>163,145</u>	<u>94,205</u>	<u>257,350</u>	<u>32,168</u>	<u>2,636</u>	<u>34,804</u>	<u>222,546</u>
		<u>\$163,145</u>	<u>\$197,281</u>	<u>\$360,426</u>	<u>\$55,703</u>	<u>\$ 3,357</u>	<u>\$59,060</u>	<u>\$301,366</u>

20. LEASES

The Hospital leases real estate for physician practices, office space, and patient service areas; these agreements typically have initial lease terms of five to 10 years. Equipment lease agreements typically have initial terms of five years. The Hospital determines if an arrangement to lease real estate and equipment is an operating or finance lease at the inception of the contract. ROU assets represent the right to use the underlying assets for the lease term, and lease liabilities represent the obligation to make lease payments arising from the leases. For lease terms greater than one year, at commencement date, lease liabilities are recognized based on the present value of lease payments over the lease term, and ROU assets are recognized as the present value of the lease payments plus initial direct costs and prepayments, less any lease incentives received. Many leases include rental escalation clauses and renewal options that are factored into estimated lease payments when appropriate.

As of June 30, 2024 and 2023, the following table summarizes the lease-related assets and liabilities recorded on the consolidated balance sheets (in thousands):

Lease-Related Assets and Liabilities	Classification on Consolidated Balance Sheet	2024	2023
Operating lease ROU assets	Other assets	<u>\$ 36,023</u>	<u>\$ 46,960</u>
Lease liabilities—current	Accounts payable and accrued expenses	\$ 14,764	\$ 14,568
Lease liabilities—noncurrent	Other noncurrent liabilities	<u>23,278</u>	<u>34,837</u>
Total operating lease liabilities		<u>\$ 38,042</u>	<u>\$ 49,405</u>

The Hospital utilized the private company accounting policy elective to use the risk-free rate to discount the lease payments. As of June 30, 2024 and 2023, the weighted-average discount rate was 1.62% and 1.25%, respectively.

As of June 30, 2024 and 2023, the weighted-average remaining lease term of all operating leases was four years and five years, respectively.

At June 30, 2024 and 2023, the components of the expenses related to leases and their classification in the consolidated statements of activities were as follows (in thousands):

Component of Lease Expense	Purchased Services	Supplies	Other Expenses	2024 Total
Operating lease expense	\$ 1,125	\$ 5,389	\$ 17,242	\$ 23,756
Variable and short-term lease expense	<u>-</u>	<u>-</u>	<u>1,264</u>	<u>1,264</u>
Total	<u>\$ 1,125</u>	<u>\$ 5,389</u>	<u>\$ 18,506</u>	25,020
Less sublease income				<u>(4,910)</u>
Total lease expense				<u>\$ 20,110</u>
Component of Lease Expense	Purchased Services	Supplies	Other Expenses	2023 Total
Operating lease expense	\$ 1,279	\$ 3,100	\$ 17,962	\$ 22,341
Variable and short-term lease expense	<u>-</u>	<u>-</u>	<u>639</u>	<u>639</u>
Total	<u>\$ 1,279</u>	<u>\$ 3,100</u>	<u>\$ 18,601</u>	22,980
Less sublease income				<u>(4,731)</u>
Total lease expense				<u>\$ 18,249</u>

At June 30, 2024 and 2023, the cash flow and other information related to leases were as follows (in thousands):

	2024	2023
Cash paid for amounts included in the measurement of lease liabilities—operating cash outflows from operating and short-term leases	\$ 17,156	\$ 15,814
ROU assets obtained in exchange for lease obligations—operating leases	453	6,400

At June 30, 2024, the future maturities of lease liabilities were as follows (in thousands):

**Years Ending
June 30**

2025	\$ 15,260
2026	11,307
2027	2,708
2028	2,213
2029	2,158
Thereafter	<u>5,927</u>
Total	39,573
Less present value discount	<u>(1,531)</u>
Lease liability	<u><u>\$ 38,042</u></u>

The following table lists minimum lease payment amounts due under sublease arrangements at June 30, 2024 (in thousands):

**Years Ending
June 30**

2025	\$ 4,949
2026	5,044
2027	4,042
2028	2,162
2029	1,814
Thereafter	<u>4,405</u>
Total	<u><u>\$ 22,416</u></u>

At June 30, 2024, there were no leases entered into that had not yet commenced.

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SUPPLEMENTAL SCHEDULE

CHILDREN'S HOSPITAL LOS ANGELES

SCHEDULE OF EXPENDITURES OF FEDERAL AWARDS
FOR THE YEAR ENDED JUNE 30, 2024

Federal Awarding Agency/ Program Title	Federal ALN Number	Additional Award Identification (Optional)	Name of Funder Pass Through Entity	Identifying Number Assigned by Pass-Through Entity	Total Amount Provided to Sub-recipient	Federal Expenditure	Federal Expenditure Subtotal by ALN
RESEARCH AND DEVELOPMENT CLUSTER							
Department of Defense:							
Military Medical Research And Development	12.420	000013939		W81XWH2110257	76,409	\$ 75,505	\$ 229,846
Military Medical Research And Development	12.420	000014235	MEMORIAL HEALTH SERVICES	W81XWH2210881		76,529	\$ 229,846
Military Medical Research And Development	12.420	000015524	USC	W81XWH2210301		4,469	\$ 229,846
Military Medical Research And Development	12.420	000015586	USC	000533783-SC008		22,724	\$ 229,846
Military Medical Research And Development	12.420	000016069		0011960271-0001		44,362	\$ 229,846
Military Medical Research And Development	12.420	RCF011603		W81XWH1910359		(86)	\$ 229,846
Military Medical Research And Development	12.420	RCF011694		W81XWH-18-1-0418		(4,270)	\$ 229,846
Military Medical Research And Development	12.420	RCZ011876	REGENTS OF THE UNIVERSITY OF CALIFORNIA	000422086-019		10,615	\$ 229,846
Military Medical Research And Development	12.420	RGP010915		516840006T		(2)	\$ 229,846
Total Department of Defense						\$ 76,409	\$ 229,846
Department Of Health And Human Services:							
Cooperative Agreements To Promote Adolescent Health Through School-based Hiv/std Prevention And School-based Surveillance	93.079	000013424		6 NU87PS004362-04-01		\$ 2,375	\$ 448,747
Cooperative Agreements To Promote Adolescent Health Through School-based Hiv/std Prevention And School-based Surveillance	93.079	000014219		6 NU87PS004362-04-01		87,953	\$ 448,747
Cooperative Agreements To Promote Adolescent Health Through School-based Hiv/std Prevention And School-based Surveillance	93.079	TGF011352		6NU87PS004362-05-04		358,419	\$ 448,747
Food And Drug Administration_research	93.103	000013461		1P50FD006425-01		105,790	\$ 1,965,774
Food And Drug Administration_research	93.103	000014293		1P50FD006425-01		17	\$ 1,965,774
Food And Drug Administration_research	93.103	000014932		75F40122C00051	591,626	983,088	\$ 1,965,774
Food And Drug Administration_research	93.103	000015516		A24-0002-S001-CHLA		237,608	\$ 1,965,774
Food And Drug Administration_research	93.103	RGF011519		1U01FD006549-01		2,142	\$ 1,965,774
Food And Drug Administration_research	93.103	RGF011721		1P50FD006425-01	74,124	637,129	\$ 1,965,774
Maternal And Child Health Federal Consolidated Programs	93.11	000012715		SUB00001875		(6,193)	\$ 3,223,528
Maternal And Child Health Federal Consolidated Programs	93.11	000012906		T72MC37404		270,778	\$ 3,223,528
Maternal And Child Health Federal Consolidated Programs	93.11	000013697	COMMUNITY HEALTH PROJECT INC	2 T73MC00008-29-00		902,553	\$ 3,223,528
Maternal And Child Health Federal Consolidated Programs	93.11	000014115		12684sc		3,901	\$ 3,223,528
Maternal And Child Health Federal Consolidated Programs	93.11	000014674		5 T71MC30799¿08¿00		418,362	\$ 3,223,528
Maternal And Child Health Federal Consolidated Programs	93.11	000014903				43,313	\$ 3,223,528
Maternal And Child Health Federal Consolidated Programs	93.11	000015005	UNIVERSITY OF SOUTHERN CALIFORNIA	14064sc		(2,857)	\$ 3,223,528
Maternal And Child Health Federal Consolidated Programs	93.11	000015341		6 T77MC25732¿12¿01		278,840	\$ 3,223,528
Maternal And Child Health Federal Consolidated Programs	93.11	000016186		PH-005258		64,968	\$ 3,223,528
Maternal And Child Health Federal Consolidated Programs	93.11	RDD004888		FUND SOURCE 234014		1,093,318	\$ 3,223,528
Maternal And Child Health Federal Consolidated Programs	93.11	RGP008083		PROGINC 340B		127,084	\$ 3,223,528
Maternal And Child Health Federal Consolidated Programs	93.11	RGP010079	FLORIDA STATE UNIVERSITY	226858		359	\$ 3,223,528
Maternal And Child Health Federal Consolidated Programs	93.11	SGP007645		CIBDIX2012HRSA-CHLA-9		9,304	\$ 3,223,528
Maternal And Child Health Federal Consolidated Programs	93.11	TGF009673		T73MC00008		(779)	\$ 3,223,528
Maternal And Child Health Federal Consolidated Programs	93.11	TGF010274		T71MC30799		(13)	\$ 3,223,528
Maternal And Child Health Federal Consolidated Programs	93.11	TGF010929		T73MC00008		(1,222)	\$ 3,223,528
Maternal And Child Health Federal Consolidated Programs	93.11	TGF011266		2 T77MC257320600		21,812	\$ 3,223,528
Environmental Health	93.113	000012884	UNIVERSITY OF SOUTHERN CALIFORNIA	5R01ES031247-05	3,582	375,896	\$ 398,319
Environmental Health	93.113	000013899	USC	3R01ES031247-02S1	2,159	1,080	\$ 398,319
Environmental Health	93.113	RGP012094		139160576		21,343	\$ 398,319

CHILDREN'S HOSPITAL LOS ANGELES

SCHEDULE OF EXPENDITURES OF FEDERAL AWARDS
FOR THE YEAR ENDED JUNE 30, 2024

Federal Awarding Agency/ Program Title	Federal ALN Number	Additional Award Identification (Optional)	Name of Funder Pass Through Entity	Identifying Number Assigned by Pass-Through Entity	Total Amount Provided to Sub-recipient	Federal Expenditure	Federal Expenditure Subtotal by ALN
RESEARCH AND DEVELOPMENT CLUSTER							
Oral Diseases And Disorders Research	93.121	000012920		10833PO 4610805-0-46		86,634	\$ 1,339,133
Oral Diseases And Disorders Research	93.121	000012943		1R01DE030286-01A1	1,279,928	1,086,909	\$ 1,339,133
Oral Diseases And Disorders Research	93.121	000013308		12731SUB		17,507	\$ 1,339,133
Oral Diseases And Disorders Research	93.121	000013735	REGENTS UNIV OF CALIFORNIA LOS ANGELES	SCON-00003429		57,610	\$ 1,339,133
Oral Diseases And Disorders Research	93.121	000013752		SUG3DE031247-01(002)		6,037	\$ 1,339,133
Oral Diseases And Disorders Research	93.121	000016168	USC	13197SUB		43,866	\$ 1,339,133
Oral Diseases And Disorders Research	93.121	RGP007781	BOARD OF TRUSTEES OF THE LELAND STANFORD	SUB:10678		7,073	\$ 1,339,133
Oral Diseases And Disorders Research	93.121	RGP007782		SUB: 11532		(11,316)	\$ 1,339,133
Oral Diseases And Disorders Research	93.121	RGP009337		SUB: 64742091		(674)	\$ 1,339,133
Oral Diseases And Disorders Research	93.121	RGP011202		SUB: 12476SUB		61	\$ 1,339,133
Oral Diseases And Disorders Research	93.121	RGP012212		10044863-14		14,661	\$ 1,339,133
Oral Diseases And Disorders Research	93.121	RGP012356		12281SUB		30,765	\$ 1,339,133
Human Genome Research	93.172	000014985	STATE UNIVERSITY OF IOWA	1R21HG012626-01A1		168,043	\$ 168,043
National Institute on Deafness and Other Communication Disorders	93.173	000014123		3K23DC014291-05S1		223	\$ 199,816
National Institute on Deafness and Other Communication Disorders	93.173	000014340	REGENTS UNIV OF CALIFORNIA LOS ANGELES	7R21DC017834-02		199,593	\$ 199,816
Research On Healthcare Costs, Quality And Outcomes	93.226	000013257	CHILDREN'S HOSPITAL MEDICAL CENTER	1R01HS027619-01A1	200,975	116,118	\$ 265,173
Research On Healthcare Costs, Quality And Outcomes	93.226	000013318		133279571		(432)	\$ 265,173
Research On Healthcare Costs, Quality And Outcomes	93.226	000014021		GENFD0002176430		113,132	\$ 265,173
Research On Healthcare Costs, Quality And Outcomes	93.226	000014277				157	\$ 265,173
Research On Healthcare Costs, Quality And Outcomes	93.226	000014831		SUBK00017769		6,775	\$ 265,173
Research On Healthcare Costs, Quality And Outcomes	93.226	000014894		CLIN2SCD-12031		3,218	\$ 265,173
Research On Healthcare Costs, Quality And Outcomes	93.226	000015659		63270494-306760		21,649	\$ 265,173
Research On Healthcare Costs, Quality And Outcomes	93.226	RCZ011883	UNIVERSITY OF CAPE TOWN	U01AI126612		4,556	\$ 265,173
Mental Health Research Grants	93.242	000013420		7R21MH119455-03		(774)	\$ 3,112,102
Mental Health Research Grants	93.242	000013422		5R01MH120133-05	17,007	302,781	\$ 3,112,102
Mental Health Research Grants	93.242	000014047		SCON-00003689		18,082	\$ 3,112,102
Mental Health Research Grants	93.242	000014266		5R01MH121462-05	922,541	708,735	\$ 3,112,102
Mental Health Research Grants	93.242	000014288		3R01MH120133-05S1		12,075	\$ 3,112,102
Mental Health Research Grants	93.242	000014450		1R56MH128427-01A1		788,366	\$ 3,112,102
Mental Health Research Grants	93.242	000014559	UNIVERSITY OF CHICAGO	1925GZA055		11,930	\$ 3,112,102
Mental Health Research Grants	93.242	000014763		CON-80005312 (GR125430)		643,735	\$ 3,112,102
Mental Health Research Grants	93.242	000014900		2022-017-CHLA-Y2		46,311	\$ 3,112,102
Mental Health Research Grants	93.242	000015142	WASHINGTON UNIVERSITY	352165		(6,078)	\$ 3,112,102
Mental Health Research Grants	93.242	000015932	UNIV OF DELAWARE	7K23MH131852-02		90,512	\$ 3,112,102
Mental Health Research Grants	93.242	RGF011590		5R01MH067842-19	242,472	461,321	\$ 3,112,102
Mental Health Research Grants	93.242	RGP009351		UNIV58459		(9)	\$ 3,112,102
Mental Health Research Grants	93.242	RGP012675				35,115	\$ 3,112,102
Alcohol Research Program	93.273	000015233		2R01AA025653-07	67,465	308,951	\$ 372,252
Alcohol Research Program	93.273	RGF010036	NATIONAL MARROW DONOR PROGRAM	5R01AA025653-05	340,452	63,301	\$ 372,252

CHILDREN'S HOSPITAL LOS ANGELES

SCHEDULE OF EXPENDITURES OF FEDERAL AWARDS
FOR THE YEAR ENDED JUNE 30, 2024

Federal Awarding Agency/ Program Title	Federal ALN Number	Additional Award Identification (Optional)	Name of Funder Pass Through Entity	Identifying Number Assigned by Pass-Through Entity	Total Amount Provided to Sub-recipient	Federal Expenditure	Federal Expenditure Subtotal by ALN
RESEARCH AND DEVELOPMENT CLUSTER							
Drug Abuse And Addiction Research Programs	93.279	000012737		3R34DA050254-01		31,589	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	000013095	SEATTLE CHILDRENS HOSPITAL	1935 G YA097		(5,877)	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	000013439	REGENTS OF THE UNIVERSITY OF CALIFORNIA	3R34DA050254-01S2		(13,585)	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	000013809	EMORY UNIVERSITY	KR 704895		48,143	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	000013888	REGENTS OF THE UNIVERSITY OF CALIFORNIA	5U01DA055362-04	72,039	1,589,187	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	000014221	BOARD OF TRUSTEES OF THE LELAND STANFORD	5U01DA036926-10		(8,872)	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	000014222	KAISER FOUNDATION HOSPITALS	5U01DA041048-08		129	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	000014658		5U01DA041048-08		(8,434)	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	000014741		KR 705044		17,987	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	000014951		5U01DA036926-08	521,608	272,229	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	000015339		704033		53,325	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	000015523	REGENTS OF THE UNIVERSITY OF CALIFORNIA	5U01DA041048-08	465,268	1,538,018	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	000015653		5U01DA036926-10	1,185,942	2,978,815	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	RGF009146		U01DA036926	(15,944)	(9,000)	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	RGF009209	CHILDRENS HOSPITAL BOSTON	SUB P20150407		(3,984)	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	RGF011158		1R34DA044106-01A1		1,262	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	RGF012526	OREGON HEALTH & SCIENCE UNIVERSITY	5U01DA036926-10		(7,582)	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	RGF012542	REGENTS UNIV OF CALIFORNIA LOS ANGELES	5U01DA041048-08		(404)	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	RGF012667		1R34DA050531-01A1	88,100	94,537	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	RGP012005		121643071 (S9002388)		54,542	\$ 6,784,496
Drug Abuse And Addiction Research Programs	93.279	RGP012034		800182		162,471	\$ 6,784,496
Centers for Disease Control and Prevention_Investigations and Technical Assistance	93.283	000013219		GENFD0002051212		(10,632)	\$ (10,632)
Discovery and Applied Research for Technological Innovations to Improve Human Health	93.286	000012699	ARIZONA STATE UNIVERSITY	3R01EB025032-03S1	104,000	69,216	\$ 128,477
Discovery and Applied Research for Technological Innovations to Improve Human Health	93.286	000013013	REGENTS OF UNIVERSITY OF COLORADO	3R01EB025032-04S1	50,957	59,374	\$ 128,477
Discovery and Applied Research for Technological Innovations to Improve Human Health	93.286	000015078				(113)	\$ 128,477
Minority Health And Health Disparities Research	93.307	000013266	CHILDREN'S NATIONAL MEDICAL CENTER	GC10358-04-DGR15294		18,460	\$ 7,024,601
Minority Health And Health Disparities Research	93.307	000014356		5P50MD017344-03	448,752	1,302,836	\$ 7,024,601
Minority Health And Health Disparities Research	93.307	000014405		5P50MD017344-03	914,384	704,577	\$ 7,024,601
Minority Health And Health Disparities Research	93.307	000014406		5P50MD017344-03	973,743	526,338	\$ 7,024,601
Minority Health And Health Disparities Research	93.307	000014408		5P50MD017344-03	732,306	1,070,134	\$ 7,024,601
Minority Health And Health Disparities Research	93.307	000014409	CHILDREN'S HOSPITAL OF PHILADELPHIA	5P50MD017344-03	2,820,409	1,519,657	\$ 7,024,601
Minority Health And Health Disparities Research	93.307	000014410	REGENTS OF THE UNIV OF MINNESOTA	5P50MD017344-03	1,469,778	835,892	\$ 7,024,601
Minority Health And Health Disparities Research	93.307	000014930		SCON-00003301, 00004325		3,213	\$ 7,024,601
Minority Health And Health Disparities Research	93.307	000014978	REGENTS UNIVERSITY OF CALIFORNIA LA	1K23MD016944-01A1		164,903	\$ 7,024,601
Minority Health And Health Disparities Research	93.307	000015019		3P50MD017344-02S1	394,762	233,338	\$ 7,024,601
Minority Health And Health Disparities Research	93.307	000015481			56,773	283,813	\$ 7,024,601
Minority Health And Health Disparities Research	93.307	000015741		3P50MD017344-04S1	61,391	85,853	\$ 7,024,601
Minority Health And Health Disparities Research	93.307	000015804		5P50MD017344-03		1,650	\$ 7,024,601
Minority Health And Health Disparities Research	93.307	000015805		5P50MD017344-03		15,270	\$ 7,024,601
Minority Health And Health Disparities Research	93.307	000015806		5P50MD017344-03		25,000	\$ 7,024,601
Minority Health And Health Disparities Research	93.307	000015807	BOARD OF TRUSTEES OF THE UNIV OF ILLINOI	3P50MD017344-02S1	216,384	143,205	\$ 7,024,601
Minority Health And Health Disparities Research	93.307	RGP010351	UNIVERSITY OF SOUTHERN CALIFORNIA	1557GV8076		90,462	\$ 7,024,601

CHILDREN'S HOSPITAL LOS ANGELES

SCHEDULE OF EXPENDITURES OF FEDERAL AWARDS
FOR THE YEAR ENDED JUNE 30, 2024

Federal Awarding Agency/ Program Title	Federal ALN Number	Additional Award Identification (Optional)	Name of Funder Pass Through Entity	Identifying Number Assigned by Pass-Through Entity	Total Amount Provided to Sub-recipient	Federal Expenditure	Federal Expenditure Subtotal by ALN
RESEARCH AND DEVELOPMENT CLUSTER							
Trans-NIH Research Support	93.31	RGP009855		1GG101662-01		(49)	\$ (49)
Rare Disorders: Research, Surveillance, Health Promotion, And Education	93.315	RGF012021		6 U01DD001274-05-01		74,019	\$ 93,217
Rare Disorders: Research, Surveillance, Health Promotion, And Education	93.315	RGF012146		6 U01DD001273-05-01		19,198	\$ 93,217
National Center For Advancing Translational Sciences	93.35	000012894		PO #50940321		(1)	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000013152		SCON-00002756		(25,464)	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000013158		133292390		4,701	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000013407	FLORIDA STATE UNIVERSITY	50910882		71	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000014842		SCON-00004232		(461)	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000014847		SCON-00004232		(1,081)	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000014914		SCON-00004232		81,582	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000015485		SCON-00004232		48,930	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000015486	REGENTS UNIV OF CALIFORNIA LOS ANGELES	SCON-00004232		53,410	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000015487		SCON-00004232		49,991	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000015488		SCON-00004232		178,288	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000015489		SCON-00004232		164,324	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000015490		SCON-00004232		13,103	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000015535		SCON-00003550		11,537	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000015536		SCON-00003550		2,938	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000015537		SCON-00003550		11,537	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000015538		SCON-00003550		111,098	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000015539		SCON-00003550		(10,878)	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000015986		PO# 00153883		145,870	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000016416	KAISER FOUNDATION HOSPITALS	SCON-00003550		3,419	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000016418		SCON-00003550		720	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000016419		SCON-00003550		31,569	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000016420		SCON-00003550		21,488	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000016424		SCON-00004232		7,121	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000016425	BRIGHAM AND WOMEN'S HOSPITAL INC	SCON-00004232		17,803	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000016426	EMORY UNIVERSITY	SCON-00004232		10,682	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000016427	WUQU KAWOQ SA	SCON-00004232		27,987	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000016428		SCON-00004232		5,543	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	000016429		SCON-00004232		3,560	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	RCC011335		Protocol S1700303		243	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	RCZ012016		12498SUB		79,900	\$ 1,143,460
National Center For Advancing Translational Sciences	93.35	RGP010611		2018-3630		93,603	\$ 1,143,460
Oxygen Contrast Imaging of Cer	93.35	RGP010151	UNIV OF NORTH CAROLINA AT CHAPEL HILL	SUB: 92097761		327	\$ 1,143,460
Research Infrastructure Programs	93.351	000014025		Equipment Grant- 8030000		249,872	\$ 329,734
Research Infrastructure Programs	93.351	000014252				79,862	\$ 329,734
Cancer Immunotherapy Trials Network Central Operations and Statistical Center - Pediatric Clinical Research Administrator Awards	93.353	000012776		GMO201108-PO00001902B		224,067	\$ 227,015
Cancer Immunotherapy Trials Network Central Operations and Statistical Center - Pediatric Clinical Research Administrator Awards	93.353	000014484	SCRIPPS HEALTH	GMO201108-PO00001902B		(6,448)	\$ 227,015
Cancer Immunotherapy Trials Network Central Operations and Statistical Center - Pediatric Clinical Research Administrator Awards	93.353	RCZ010731	CHILDREN'S HOSPITAL OF PHILADELPHIA	PBTC-042 UM1CA81457		9,396	\$ 227,015

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RESEARCH AND DEVELOPMENT CLUSTER							
Nursing Research	93.361	000013508	UNIV OF CALIFORNIA SAN FRANCISCO			142,888	\$ 225,625
Nursing Research	93.361	RGP009730		229712		4,008	\$ 225,625
Nursing Research	93.361	RGP011275		3201511123 / PO# 20043698		78,729	\$ 225,625
Cancer Cause And Prevention Research	93.393	000013025		142125127		14,720	\$ 511,744
Cancer Cause And Prevention Research	93.393	000013740		PO1278328		133,520	\$ 511,744
Cancer Cause And Prevention Research	93.393	000013918		R01CA251112-01		4,724	\$ 511,744
Cancer Cause And Prevention Research	93.393	000014113		PO# 3000239767		99,585	\$ 511,744
Cancer Cause And Prevention Research	93.393	000014562		2021-1522		(12,142)	\$ 511,744
Cancer Cause And Prevention Research	93.393	000015098		PID #835641 PO #25660569		48,708	\$ 511,744
Cancer Cause And Prevention Research	93.393	000016328		2021-1522		11,594	\$ 511,744
Cancer Cause And Prevention Research	93.393	RGF011591	BOARD OF TRUSTEES OF THE UNIV OF ILLINOI	5R01CA137124-11	16,875	211,538	\$ 511,744
Cancer Cause And Prevention Research	93.393	RGF012638		4R00CA197484-03		41,543	\$ 511,744
Cancer Cause And Prevention Research	93.393	RGP011514				(42,046)	\$ 511,744
Cancer Detection And Diagnosis Research	93.394	000013075		SCON-00002188		132,919	\$ 1,580,499
Cancer Detection And Diagnosis Research	93.394	000013980		WU-24-0407		70,055	\$ 1,580,499
Cancer Detection And Diagnosis Research	93.394	000014098		1308004		5,421	\$ 1,580,499
Cancer Detection And Diagnosis Research	93.394	000014945		1R01CA274137-01A1	85,867	884,921	\$ 1,580,499
Cancer Detection And Diagnosis Research	93.394	000015165		5R01CA282759-02	87,584	383,565	\$ 1,580,499
Cancer Detection And Diagnosis Research	93.394	000016204		GRT-00002960 -PO#20507524		103,618	\$ 1,580,499
Cancer Treatment Research	93.395	000013272		1R37CA252236-01A1		392,250	\$ 1,845,286
Cancer Treatment Research	93.395	000013514	JOHNS HOPKINS UNIVERSITY	5R21CA262978-02		6,151	\$ 1,845,286
Cancer Treatment Research	93.395	000013532		A22-0004-S003		98,167	\$ 1,845,286
Cancer Treatment Research	93.395	000013563		PO# 20214077/GRT-00000636		(6,469)	\$ 1,845,286
Cancer Treatment Research	93.395	000013564		PO# 20214077/GRT-00000636		6,344	\$ 1,845,286
Cancer Treatment Research	93.395	000013565		PO# 20214077/GRT-00000636		(3,330)	\$ 1,845,286
Cancer Treatment Research	93.395	000013566		PO# 20214077/GRT-00000636		49	\$ 1,845,286
Cancer Treatment Research	93.395	000013654		PO# 20214077/GRT-00000636		2,826	\$ 1,845,286
Cancer Treatment Research	93.395	000013730		FP00026529 SUB681		2,450	\$ 1,845,286
Cancer Treatment Research	93.395	000014251		RESEARCH INSTITUTE AT NATIONWIDE		(359)	\$ 1,845,286
Cancer Treatment Research	93.395	000014430		UNIV OF NORTH CAROLINA AT CHAPEL HILL		10,942	\$ 1,845,286
Cancer Treatment Research	93.395	000014452	USC	13834		159,092	\$ 1,845,286
Cancer Treatment Research	93.395	000014486				68,951	\$ 1,845,286
Cancer Treatment Research	93.395	000014487	BATTELLE MEMORIAL INSTITUTE			181,903	\$ 1,845,286
Cancer Treatment Research	93.395	000014529		30006717-01		11,150	\$ 1,845,286
Cancer Treatment Research	93.395	000014530	UNIVERSITY OF SOUTHERN CALIFORNIA	30006716-01		(65)	\$ 1,845,286
Cancer Treatment Research	93.395	000015245		SUBK00016732		51,120	\$ 1,845,286
Cancer Treatment Research	93.395	000015393	KAISER FOUNDATION HOSPITALS	3U10CA180886-09S1		94,193	\$ 1,845,286
Cancer Treatment Research	93.395	000015412		1334806		95,011	\$ 1,845,286
Cancer Treatment Research	93.395	000015415	USC	U10CA180886		420	\$ 1,845,286
Cancer Treatment Research	93.395	000015550		SUM1CA228823-05		(8,021)	\$ 1,845,286
Cancer Treatment Research	93.395	000015556		000015556		141,516	\$ 1,845,286

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RESEARCH AND DEVELOPMENT CLUSTER							
Cancer Treatment Research	93.395	RCZ008632		110068190-77		57,618	\$ 1,845,286
Cancer Treatment Research	93.395	RCZ010962		SUB: 1100681907761550		37,075	\$ 1,845,286
Cancer Treatment Research	93.395	RCZ012248	USC	FP00025505_SUB01_AR67455		69,439	\$ 1,845,286
Cancer Treatment Research	93.395	RGF009394	CHILDREN'S HOSPITAL CORPORATION	R01CA181050	(566)	0	\$ 1,845,286
Cancer Treatment Research	93.395	RGF011774	OREGON HEALTH & SCIENCE UNIVERSITY	2R01CA172896-06		101,968	\$ 1,845,286
Cancer Treatment Research	93.395	RGPO08993		5401		63,536	\$ 1,845,286
Cancer Treatment Research	93.395	RGPO09001	UNIVERSITY OF TURIN	961558-RSUB		(17,219)	\$ 1,845,286
Cancer Treatment Research	93.395	RGPO09834		FP00013013S		189,937	\$ 1,845,286
Cancer Treatment Research	93.395	RGPO09837		SUB: 021119		59,008	\$ 1,845,286
Cancer Treatment Research	93.395	RGPO10917		FP00015221		13,488	\$ 1,845,286
Cancer Treatment Research	93.395	RGPO10998		11993SUB		(30,724)	\$ 1,845,286
Cancer Treatment Research	93.395	RGPO11242		1645 G VA144		832	\$ 1,845,286
Cancer Treatment Research	93.395	RGPO11507				(1,826)	\$ 1,845,286
Cancer Treatment Research	93.395	RGPO12044		11909SUB		(2,137)	\$ 1,845,286
Cancer Biology Research	93.396	000014101		25914-02-386		31,643	\$ 43,493
Cancer Biology Research	93.396	RGF010824	WASHINGTON STATE UNIVERSITY	SUB: RGF10824A		11,850	\$ 43,493
Cancer Centers Support Grants	93.397	000012738		GMO 200902 PO#000001843A		(63,458)	\$ 1,000,263
Cancer Centers Support Grants	93.397	000013087		129806534		49,489	\$ 1,000,263
Cancer Centers Support Grants	93.397	000013381		75839370		582	\$ 1,000,263
Cancer Centers Support Grants	93.397	000013546	CALIFORNIA STATE UNIVERSITY CHANNEL	SCON-00004200		23,158	\$ 1,000,263
Cancer Centers Support Grants	93.397	000013906	USC	GMO221202 PO000002522B		285,376	\$ 1,000,263
Cancer Centers Support Grants	93.397	000014455		SCON-00004200		8,401	\$ 1,000,263
Cancer Centers Support Grants	93.397	000014456		SCON-00004200		135,012	\$ 1,000,263
Cancer Centers Support Grants	93.397	000014457		SCON-00004200		152,881	\$ 1,000,263
Cancer Centers Support Grants	93.397	000014458	SAN DIEGO STATE UNIVERSITY FOUNDATION	SCON-00004200		107,602	\$ 1,000,263
Cancer Centers Support Grants	93.397	000014459	SANSUM DIABETES RESEARCH INSTITUTE	SCON-00004200		155,947	\$ 1,000,263
Cancer Centers Support Grants	93.397	000015657		SCON-00004200		89,078	\$ 1,000,263
Cancer Centers Support Grants	93.397	RGPO10193		75839370		55,528	\$ 1,000,263
Cancer Centers Support Grants	93.397	RGPO10208		75839370		(547)	\$ 1,000,263
Cancer Centers Support Grants	93.397	RGPO12156		75839370		1,706	\$ 1,000,263
Cancer Centers Support Grants	93.397	RGPO12502		75839370		(492)	\$ 1,000,263
Cancer Research Manpower	93.398	000014031		K08CA237868		196,081	\$ 587,239
Cancer Research Manpower	93.398	000015750		5K99CA270282-02		103,398	\$ 587,239
Cancer Research Manpower	93.398	TGF011511		1R25CA225513-01A1	27,324	269,174	\$ 587,239
Cancer Research Manpower	93.398	TGF011638		5K08CA232344-05		18,586	\$ 587,239
Cancer Control	93.399	000013462		P.O.# P700000777		46,681	\$ 58,679
Cancer Control	93.399	000015504		5UG1CA189955-09		813	\$ 58,679
Cancer Control	93.399	000015552		P700000330	11,000	11,185	\$ 58,679
NIH-NHLBI	93.837	000012718		5K01HL153942-05		117,979	\$ 2,170,482
NIH-NHLBI	93.837	000012751	ICAHN SCHOOL OF MEDICINE AT MOUNT SINAI	1R01HL148706-01A1		533,766	\$ 2,170,482
NIH-NHLBI	93.837	000012800		CON-80005346 (GR125722)		5,036	\$ 2,170,482

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RESEARCH AND DEVELOPMENT CLUSTER							
NIH-NHLBI	93.837	000012961	ARIZONA STATE UNIVERSITY			235,259	\$ 2,170,482
NIH-NHLBI	93.837	000013039	REGENTS UNIVERSITY OF CALIFORNIA LA	1564GYA698		20,753	\$ 2,170,482
NIH-NHLBI	93.837	000013280		SCON-00003767		135,652	\$ 2,170,482
NIH-NHLBI	93.837	000013789		1K23HL161310-01		180,792	\$ 2,170,482
NIH-NHLBI	93.837	000013805	BRIGHAM AND WOMEN'S HOSPITAL INC	5K25HL153954-04		192,675	\$ 2,170,482
NIH-NHLBI	93.837	000013990		GRT-00002339-0724		20,821	\$ 2,170,482
NIH-NHLBI	93.837	000014034		901632-CHLA		231,287	\$ 2,170,482
NIH-NHLBI	93.837	000014366				(374)	\$ 2,170,482
NIH-NHLBI	93.837	000014569				259,194	\$ 2,170,482
NIH-NHLBI	93.837	000014888	UNIVERSITY OF UTAH	2021 PCGC 3313945/OS00026		50,001	\$ 2,170,482
NIH-NHLBI	93.837	000014891				10,787	\$ 2,170,482
NIH-NHLBI	93.837	000015063	USC			24,002	\$ 2,170,482
NIH-NHLBI	93.837	000015534		3K23HL155898-03S1		68,313	\$ 2,170,482
NIH-NHLBI	93.837	000015939	WASHINGTON UNIVERSITY	VUMC109446		1,271	\$ 2,170,482
NIH-NHLBI	93.837	RCZ009398		PHN SVRIII		(13,016)	\$ 2,170,482
NIH-NHLBI	93.837	RGF010339	UNIV OF CALIFORNIA SAN FRANCISCO	SUBS: RGF010339 A & B	174,435	82,446	\$ 2,170,482
NIH-NHLBI	93.837	RGF010515	UNIVERSITY OF PITTSBURGH	1R01HL130172		2,107	\$ 2,170,482
NIH-NHLBI	93.837	RGF011524	TRUSTEES OF TUFTS COLLEGE	5R01HL145641-04		(180)	\$ 2,170,482
NIH-NHLBI	93.837	RGP010713		GR103094		(9,437)	\$ 2,170,482
NIH-NHLBI	93.837	RGP011637		FP00001653 PCGC Downs sup		(13,419)	\$ 2,170,482
NIH-NHLBI	93.837	RGP011989		0046317 (126441-7)		13,250	\$ 2,170,482
NIH-NHLBI	93.837	SGP011913		138281		21,517	\$ 2,170,482
Lung Diseases Research	93.838	000013002	CHILDREN'S NATIONAL MEDICAL CENTER	3R01HL134666-04S1	50,443	231,651	\$ 3,603,085
Lung Diseases Research	93.838	000013149		SA0000802		16,543	\$ 3,603,085
Lung Diseases Research	93.838	000013276	USC	5K23HL153756-04		170,874	\$ 3,603,085
Lung Diseases Research	93.838	000013619		MHS-20NIVMASK1-14		20,612	\$ 3,603,085
Lung Diseases Research	93.838	000013901		1R01HL151699-01A1		6,239	\$ 3,603,085
Lung Diseases Research	93.838	000014237	REGENTS OF THE UNIVERSITY OF CALIFORNIA	Sub 14-01, PO M210471893		8,449	\$ 3,603,085
Lung Diseases Research	93.838	000014413	REGENTS UNIV OF CALIFORNIA LOS ANGELES			511,511	\$ 3,603,085
Lung Diseases Research	93.838	000014425	SEATTLE CHILDRENS HOSPITAL			684,793	\$ 3,603,085
Lung Diseases Research	93.838	000014432	UNIVERSITY OF WASHINGTON	OS00000264/400186		128,790	\$ 3,603,085
Lung Diseases Research	93.838	000014600		PATHO-PH2-SUB_05 _23		130,157	\$ 3,603,085
Lung Diseases Research	93.838	000014605	REGENTS OF THE UNIVERSITY OF CALIFORNIA			574,998	\$ 3,603,085
Lung Diseases Research	93.838	000014758	USC	30-1		592	\$ 3,603,085
Lung Diseases Research	93.838	000014791		KR 705112		170,935	\$ 3,603,085
Lung Diseases Research	93.838	000014818				17,600	\$ 3,603,085
Lung Diseases Research	93.838	000014913		GRT-00001474		3,980	\$ 3,603,085
Lung Diseases Research	93.838	000014946		1R01HL164397-01A1		399,245	\$ 3,603,085
Lung Diseases Research	93.838	000015877	REGENTS OF THE UNIVERSITY OF MICHIGAN	1K23HL169902-01		132,724	\$ 3,603,085
Lung Diseases Research	93.838	RGF008440		P2013-0606_USC_01		(24,433)	\$ 3,603,085
Lung Diseases Research	93.838	RGF010323	REGENTS OF THE UNIVERSITY OF CALIFORNIA	5R01HL134666-05	47,725	416,365	\$ 3,603,085
Lung Diseases Research	93.838	RGF010762		1R01HL141856-01		2,229	\$ 3,603,085
Lung Diseases Research	93.838	RGP011093				(770)	\$ 3,603,085

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RESEARCH AND DEVELOPMENT CLUSTER							
Blood Diseases And Resources Research	93.839	000014563	USC	A907285 formerly A767456		90,834	\$ 117,199
Blood Diseases And Resources Research	93.839	000015084		A907285 formerly A767456		11,178	\$ 117,199
Blood Diseases And Resources Research	93.839	RCC008981	ARIZONA STATE UNIVERSITY	ACRI 12-001		0	\$ 117,199
Blood Diseases And Resources Research	93.839	RGF010602	UNIVERSITY OF UTAH	1R01HL136484-01A1		24,601	\$ 117,199
Blood Diseases And Resources Research	93.839	RGP010541		1R01HL131731-01A1		(9,414)	\$ 117,199
Arthritis, Musculoskeletal And Skin Diseases Research	93.846	000013054		Advarra-19-00586		15,362	\$ 28,225
Arthritis, Musculoskeletal And Skin Diseases Research	93.846	000013172		901634-CHLA		2,000	\$ 28,225
Arthritis, Musculoskeletal And Skin Diseases Research	93.846	000014583		10061509-01-CHOL		6,782	\$ 28,225
Arthritis, Musculoskeletal And Skin Diseases Research	93.846	RGP011566		901575-CHLA		4,081	\$ 28,225
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000012745		5R01DK123234-05	277,414	422,197	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000013058	ARIZONA STATE UNIVERSITY	51511		37,908	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000013064		901628-CHLA		8,850	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000013097	UNIVERSITY OF PITTSBURGH	0255-A946-4609		11,034	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000013405		000013405		15	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000013440	USC	5R01DK126085-02	418,273	892,339	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000013695	CHILDREN'S HOSPITAL CORPORATION	5K01DK131390-03		167,182	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000013869		5R01DK110793-09	126,719	944,482	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000013912				51,116	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000013950		SCON-00004235		(30,215)	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000014001	UNIVERSITY OF CINCINNATI	3R01DK126085-01A1S1		9,016	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000014006		OS00000564/317894		15,268	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000014102		10062158-03-CHLA		115,524	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000014202		A22-2003-S02/PO # 965154		42,186	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000014382		5R01DK133459-03	276,935	579,931	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000014396		1R01DK133504-01	180,841	451,876	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000014539		51512		239,218	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000014650		1K23DK134801-01	(1,859)	183,189	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000014661		14039sc		127,452	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000014662		CON-80004372(GR118902)		1,833	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000014708		5U01DK134984-02		78,179	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000014918		1K01DK135847-01		129,457	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000015121	REGENTS OF THE UNIVERSITY OF MICHIGAN	5R01DK133459-03		125,082	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000015315		000013405-2		189,814	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000015460		1K26DK138380-01		74,442	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000015529				63,530	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000015784	REGENTS UNIVERSITY OF CALIFORNIA LA	97604-1182891-2		5,647	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	000016307		SCON-00004235		13,125	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	RCX003941	BOARD OF TRUSTEES OF THE UNIV OF ILLINOI	Protocol: 106320		40,077	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	RGF010633		7R01DK100314-03		1,988	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	RGF010786		5R01DK084142-10	246,297	453,162	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	RGF011634		2R01DK095004-06A1	38,934	71,734	\$ 6,629,450

CHILDREN’S HOSPITAL LOS ANGELES

SCHEDULE OF EXPENDITURES OF FEDERAL AWARDS
FOR THE YEAR ENDED JUNE 30, 2024

Federal Awarding Agency/ Program Title	Federal ALN Number	Additional Award Identification (Optional)	Name of Funder Pass Through Entity	Identifying Number Assigned by Pass-Through Entity	Total Amount Provided to Sub-recipient	Federal Expenditure	Federal Expenditure Subtotal by ALN
RESEARCH AND DEVELOPMENT CLUSTER							
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	RGF011684	UNIVERSITY OF ARIZONA	R01DK109161		(59,074)	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	RGF011685		5R01DK110793-05	48,608	40,528	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	RGF011799	REGENTS OF UNIVERSITY OF COLORADO	2U01DK084538-11		325,139	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	RGF011923		1R01DK119694-01A1	2,709	70,979	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	RGF012132		1R01DK121037-01A1	12,168	532,134	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	RGP010301		14D03		(58)	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	RGP010714		115956689		43,806	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	RGP011082		SUB: 91020523		(2,307)	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	TGF010062		R25DK078385		3	\$ 6,629,450
Diabetes, Digestive, And Kidney Diseases Extramural Research	93.847	TGF011624		5K08DK123377-05		161,662	\$ 6,629,450
Extramural Research Programs In The Neurosciences And Neurological Disorders	93.853	000012753	UNIVERSITY OF SOUTHERN CALIFORNIA	6(GG015970-05)		75,765	\$ 1,324,123
Extramural Research Programs In The Neurosciences And Neurological Disorders	93.853	000013147		7R01NS101029-04	37,718	(12,374)	\$ 1,324,123
Extramural Research Programs In The Neurosciences And Neurological Disorders	93.853	000013150		7R01NS095979-06		(214)	\$ 1,324,123
Extramural Research Programs In The Neurosciences And Neurological Disorders	93.853	000013426	EMORY UNIVERSITY	2004747660		20,914	\$ 1,324,123
Extramural Research Programs In The Neurosciences And Neurological Disorders	93.853	000013915	ICAHN SCHOOL OF MEDICINE AT MOUNT SINAI	10050533-15/PO#U000396886		81,440	\$ 1,324,123
Extramural Research Programs In The Neurosciences And Neurological Disorders	93.853	000014591	SONOMA TECHNOLOGY INC	SAPOG16850_ 19GG015970-06		147,033	\$ 1,324,123
Extramural Research Programs In The Neurosciences And Neurological Disorders	93.853	000014628	KAISER FOUNDATION HOSPITALS	GRT-00002996/PO# 20475646		56,035	\$ 1,324,123
Extramural Research Programs In The Neurosciences And Neurological Disorders	93.853	000014717		1K08NS125175-01A1		192,671	\$ 1,324,123
Extramural Research Programs In The Neurosciences And Neurological Disorders	93.853	000014733		SCON-00004706		42,985	\$ 1,324,123
Extramural Research Programs In The Neurosciences And Neurological Disorders	93.853	000014737		7R01NS101029-04	52,753	45,305	\$ 1,324,123
Extramural Research Programs In The Neurosciences And Neurological Disorders	93.853	000015144		SCON-00005175		144,957	\$ 1,324,123
Extramural Research Programs In The Neurosciences And Neurological Disorders	93.853	000015279		081R01NS117430-01A1		454,751	\$ 1,324,123
Extramural Research Programs In The Neurosciences And Neurological Disorders	93.853	000015496		1R03NS131977-01A1		17,850	\$ 1,324,123
Extramural Research Programs In The Neurosciences And Neurological Disorders	93.853	RCZ009383		SUB: 9797SC		60,902	\$ 1,324,123
Extramural Research Programs In The Neurosciences And Neurological Disorders	93.853	RCZ011214		SUB: 9797SC		(4,254)	\$ 1,324,123
Extramural Research Programs In The Neurosciences And Neurological Disorders	93.853	RGP011234		0050115 (131442-5)		357	\$ 1,324,123
Allergy, Immunology And Transplantation Research	93.855	000012878		11738sc		(5,837)	\$ 2,946,537
Allergy, Immunology And Transplantation Research	93.855	000013273	SEATTLE CHILDRENS HOSPITAL	5R01AI152068-02		412,738	\$ 2,946,537
Allergy, Immunology And Transplantation Research	93.855	000013425		0000191445		464,722	\$ 2,946,537
Allergy, Immunology And Transplantation Research	93.855	000013549		11738sc		82,638	\$ 2,946,537
Allergy, Immunology And Transplantation Research	93.855	000013741		62614312-128779		4,207	\$ 2,946,537
Allergy, Immunology And Transplantation Research	93.855	000014437	WASHINGTON UNIVERSITY	11-1075-7116-5792		65,166	\$ 2,946,537
Allergy, Immunology And Transplantation Research	93.855	000014572	USC	GENFD0002260194		3,329	\$ 2,946,537
Allergy, Immunology And Transplantation Research	93.855	000014584	REGENTS OF UNIVERSITY OF COLORADO	SUB00003642		183,285	\$ 2,946,537
Allergy, Immunology And Transplantation Research	93.855	000014623		5R01AI173194-02		(43,290)	\$ 2,946,537
Allergy, Immunology And Transplantation Research	93.855	000014859	SEATTLE CHILDRENS HOSPITAL	20123802-CHLA		48,235	\$ 2,946,537
Allergy, Immunology And Transplantation Research	93.855	000015470	UNIVERSITY OF CAPE TOWN	1625 G IA799		255,719	\$ 2,946,537
Allergy, Immunology And Transplantation Research	93.855	RGF010603		5U01AI126612-05	1,664,599	1,491,200	\$ 2,946,537
Allergy, Immunology And Transplantation Research	93.855	RGP008799	UNIVERSIDAD PERUANA CAYETANO HEREDIA	SUB: 8405sc		(287)	\$ 2,946,537
Allergy, Immunology And Transplantation Research	93.855	RGP011522		112525040-8078168		(15,288)	\$ 2,946,537

CHILDREN’S HOSPITAL LOS ANGELES

SCHEDULE OF EXPENDITURES OF FEDERAL AWARDS
FOR THE YEAR ENDED JUNE 30, 2024

Federal Awarding Agency/ Program Title	Federal ALN Number	Additional Award Identification (Optional)	Name of Funder Pass Through Entity	Identifying Number Assigned by Pass-Through Entity	Total Amount Provided to Sub-recipient	Federal Expenditure	Federal Expenditure Subtotal by ALN
RESEARCH AND DEVELOPMENT CLUSTER							
Child Health And Human Development Extramural Research	93.865	000012790		307942 / P.O.# 3100663968		(55,893)	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000013029		700171-0722-00		2,155	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000013077		134692388		4,066	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000013128	UNIVERSITY OF UTAH	GRT-00001533/PO 20517426		9,112	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000013292		700171-0720-00		(2,788)	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000013316		GRT-00000230 PO20231731		156,800	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000013317	EMORY UNIVERSITY	7R21HD096521-03		(70)	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000013335		30005566-02		52,600	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000013367		7R03HD096137-03		2,889	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000013466		MHS22-ETTSecurement2-11		82,450	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000013510		1R21HD101977-01		1,512	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000013556		5R01HD101641-03	144,387	573,991	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000013656	ANN & ROBERT H. LURIE CHILDREN'S	5R01HD107687-03	28,249	789,115	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000013763		5R01HD082554-09	760,312	1,094,902	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000013820	WUQU KAWOQ SA	5R33HD107983-04	44,243	66,944	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000013927		GRT-00000762/PO# 20241150		23,592	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000014037	FLORIDA STATE UNIVERSITY	SCON-00004591		22,273	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000014039		579920 - 50716		17,191	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000014111		5R01HD105656-03	461,397	660,402	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000014206		5R01HD090138-07	171,627	104,328	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000014417	ROYAL INSTITUTION FOR THE ADVANCEMENT OF	13528s c		2,548	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000014448		FY23.1039.002		416,296	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000014489		1005886001/PO U000390326		15,452	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000014510		1557 G LB501		14,236	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000014593	USC	PO#20310807/A#GRT000000762		17,781	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000014714		1UM2HD111076-01-SA003		373,545	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000014800		5124546		81	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000014839		5RL1HD107785-02		172,708	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000015251		1R03HD110889-01A1	3,562	6,091	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000015316		A24-0049-S001		12,177	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000015345		FP00019547_SA002		2,761	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	000015976		5R33HD107983-04	119,670	148,817	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	RGF008963		R01HD081197		(26,134)	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	RGF009152	CENTRE HOSPITALIER UNIVERSITAIRE SAINTE	R01HD082554		(33,639)	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	RGF011336		1R21HD097761-01A1	6,600	(416)	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	RGF011717	USC	1R01HD095456-05		264,081	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	RGF012063	REGENTS OF THE UNIVERSITY OF CALIFORNIA	5R01HD100542-04	56,669	32,430	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	RGF012543		5R13HD102137-02		30	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	RGP010625		5117718		17,433	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	RGP010857		GMO1 60101/PO# 000000771E		(1,169)	\$ 5,497,523
Child Health And Human Development Extramural Research	93.865	RGP011019		SUB: 1644GVA075		8,217	\$ 5,497,523

CHILDREN’S HOSPITAL LOS ANGELES

SCHEDULE OF EXPENDITURES OF FEDERAL AWARDS
FOR THE YEAR ENDED JUNE 30, 2024

Federal Awarding Agency/ Program Title	Federal ALN Number	Additional Award Identification (Optional)	Name of Funder Pass Through Entity	Identifying Number Assigned by Pass-Through Entity	Total Amount Provided to Sub-recipient	Federal Expenditure	Federal Expenditure Subtotal by ALN	
RESEARCH AND DEVELOPMENT CLUSTER								
Child Health And Human Development Extramural Research	93.865	RGP011097	MEDICAL COLLEGE OF WISCONSIN INC	GSNFD0001752110		3,713	\$ 5,497,523	
Child Health And Human Development Extramural Research	93.865	RGP011334		PO#203405445		116,454	\$ 5,497,523	
Child Health And Human Development Extramural Research	93.865	RGP011580		700196-0420-00		76	\$ 5,497,523	
Child Health And Human Development Extramural Research	93.865	RGP011811		60055120 CHLA		10,354	\$ 5,497,523	
Child Health And Human Development Extramural Research	93.865	RGP012537		CHI-267654-02/PO 68592578		147,625	\$ 5,497,523	
Child Health And Human Development Extramural Research	93.865	TGF011910		K23HD099309		170,404	\$ 5,497,523	
Aging Research	93.866	000013768		142621931 - SCON-00006732		23,325	\$ 732,312	
Aging Research	93.866	000014298		RAWD000066-SUB00132		362,132	\$ 732,312	
Aging Research	93.866	000014368		SCON-00005211		268,502	\$ 732,312	
Aging Research	93.866	000015229		RAWD000066-SUB00132		78,353	\$ 732,312	
Vision Research	93.867	000014056		5K23EY033790-03		242,589	\$ 486,154	
Vision Research	93.867	RCC010367		5U10EY011751-20		13,136	\$ 486,154	
Vision Research	93.867	RGF009411		R01EY026661		163	\$ 486,154	
Vision Research	93.867	RGP010247		79612331		(2,066)	\$ 486,154	
Vision Research	93.867	TGF012129		5K08EY030924-05		232,332	\$ 486,154	
National Library of Medicine	93.879	000013999		SCON-00003673		23,487	\$ 23,487	
Total Department Of Health And Human Services					\$ 19,982,495	\$ 56,624,611		
TOTAL RESEARCH AND DEVELOPMENT CLUSTER					\$ 20,058,904	\$ 56,854,457		
HEALTH CENTER PROGRAM CLUSTER								
Department Of Health And Human Services:								
Consolidated Health Centers (community Health Centers, Migrant Health Centers, Health Care For The H	93.224	000014931	H80CS00139			\$ 231,201	\$ 398,187	
Consolidated Health Centers (community Health Centers, Migrant Health Centers, Health Care For The H	93.224	000016142				167,011	\$ 398,187	
Consolidated Health Centers (community Health Centers, Migrant Health Centers, Health Care For The H	93.224	SGP012121				(25)	\$ 398,187	
Total Department Of Health And Human Services					\$ 0	\$ 398,187		
TOTAL HEALTH CENTER PROGRAM CLUSTER					\$ 0	\$ 398,187		
PROGRAMS NOT CLUSTERED:								
Department Of Health And Human Services:								
Blood Disorder Program: Prevention, Surveillance, And Research	93.08	000013588	SOUTH CENTRAL PREVENTION COALITION	CIBDIX2020CDC-CHLA-03		\$ 42,201	\$ 42,201	
Affordable Care Act (aca) Personal Responsibility Education Program	93.092	000014172		90AP2696	154,628	675,390	\$ 675,390	
Emergency Medical Services For Children	93.127	000014468		13260SUB		14,227	\$ 122,648	
Emergency Medical Services For Children	93.127	000015514		13260SUB		108,421	\$ 122,648	
Coordinated Services And Access To Research For Women, Infants, Children, And Youth	93.153	000015202		1650 G LA157		71,156	\$ 71,156	
Health Education Training Centers Continuing Educational Support For Health Professionals Serving In Underserved Communities	93.189	000012972	IN THE MEANTIME MEN'S GROUP INC	K08HS26503		(1,677)	\$ (1,677)	
Graduate Psychology Education Programs	93.191	000014517		5 D40HP33363&06&00		469,740	\$ 469,740	
Substance Abuse And Mental Health Services_projects Of Regional And National Significance	93.243	000012875		5H79SM082875-05		361,202	\$ 1,792,035	
Substance Abuse And Mental Health Services_projects Of Regional And National Significance	93.243	000013760		V2022-1	66,706	213,560	\$ 1,792,035	
Substance Abuse And Mental Health Services_projects Of Regional And National Significance	93.243	000014803			57,134	534,020	\$ 1,792,035	
Substance Abuse And Mental Health Services_projects Of Regional And National Significance	93.243	000014808		5H79SP083278-03	67,587	236,969	\$ 1,792,035	
Substance Abuse And Mental Health Services_projects Of Regional And National Significance	93.243	000014840				2,470	\$ 1,792,035	
Substance Abuse And Mental Health Services_projects Of Regional And National Significance	93.243	SGF011354		1H79TI081272-01		203,165	\$ 1,792,035	
Substance Abuse And Mental Health Services_projects Of Regional And National Significance	93.243	SGF012204		6H79SP081677-05M001		240,729	\$ 1,792,035	
Substance Abuse And Mental Health Services_projects Of Regional And National Significance	93.243	TGP010582		SUB: 165603610213		(80)	\$ 1,792,035	

CHILDREN’S HOSPITAL LOS ANGELES

SCHEDULE OF EXPENDITURES OF FEDERAL AWARDS
FOR THE YEAR ENDED JUNE 30, 2024

Federal Awarding Agency/ Program Title	Federal ALN Number	Additional Award Identification (Optional)	Name of Funder Pass Through Entity	Identifying Number Assigned by Pass-Through Entity	Total Amount Provided to Sub-recipient	Federal Expenditure	Federal Expenditure Subtotal by ALN
RESEARCH AND DEVELOPMENT CLUSTER							
Immunizations CoAg and Vaccines for Children Program previously published as Immunization Grants and Vaccines for Children Program	93.268	000014707		PH-004716		95,400	\$ 95,400
Teenage Pregnancy Prevention Program	93.297	000015640	LOS ANGELES YOUTH NETWORK	5 TP2AH000096-02-00		569,554	\$ 569,554
Public Health Service Evaluation Funds	93.343	000013363			50,000	89,830	\$ 89,830
Nurse Education, Practice Quality And Retention Grants	93.359	TGP011640				3,451	\$ 3,451
Congressional Directives	93.493	000015118		CE1HS47178		1,147,701	\$ 2,849,216
Congressional Directives	93.493	000015212				264,694	\$ 2,849,216
Congressional Directives	93.493	000015690		Fund 1020		1,436,821	\$ 2,849,216
University Centers For Excellence In Developmental Disabilities Education, Research, And Service	93.632	000013916	USC	90DDC50063-01-00		(116)	\$ 626,346
University Centers For Excellence In Developmental Disabilities Education, Research, And Service	93.632	000014267				47	\$ 626,346
University Centers For Excellence In Developmental Disabilities Education, Research, And Service	93.632	000014778	IN THE MEANTIME MEN'S GROUP INC	90UCPH0035		11,530	\$ 626,346
University Centers For Excellence In Developmental Disabilities Education, Research, And Service	93.632	000014790		90DDUC0098-03-01		608,409	\$ 626,346
University Centers For Excellence In Developmental Disabilities Education, Research, And Service	93.632	000015710		90DDUC0098-03-01		2,763	\$ 626,346
University Centers For Excellence In Developmental Disabilities Education, Research, And Service	93.632	SGF010830		90DDUC0019		3,713	\$ 626,346
Mental And Behavioral Health Education And Training Grants	93.732	000014387				376,664	\$ 506,678
Mental And Behavioral Health Education And Training Grants	93.732	000014793	LOS ANGELES YOUTH NETWORK			130,014	\$ 506,678
Children's Health Insurance Program	93.767	000015401			18,240	193,181	\$ 193,181
Section 223 Demonstration Programs to Improve Community Mental Health Services	93.829	000015691		1H79TI086977-01		215,215	\$ 215,215
Primary Care Training And Enhancement	93.884	000015300	PARA LOS NINOS	5 TA2HP48939¿03¿00	66,384	532,002	\$ 532,002
National Bioterrorism Hospital Preparedness	93.889	8887-Bioterrorism				612,156	\$ 612,156
Hiv Emergency Relief Project Grants	93.914	000013300		PH-004294		8,704	\$ 86,488
Hiv Emergency Relief Project Grants	93.914	000016362		PH-004294		5,538	\$ 86,488
Hiv Emergency Relief Project Grants	93.914	SCG011744		PH-003744		72,246	\$ 86,488
Hiv Prevention Activities Health Department Based	93.94	000015763		PH-003436		211,012	\$ 680,374
Hiv Prevention Activities Health Department Based	93.94	SCS009399		PH-003436	70,550	469,362	\$ 680,374
Block Grants For Prevention And Treatment Of Substance Abuse	93.959	000014954		PH-004377		356,964	\$ 880,961
Block Grants For Prevention And Treatment Of Substance Abuse	93.959	SCG012415		H-707261/PH-003826		524,074	\$ 880,961
Block Grants For Prevention And Treatment Of Substance Abuse	93.959	SCG012432		PH-002767		8	\$ 880,961
Block Grants For Prevention And Treatment Of Substance Abuse	93.959	SCG012448		PH-003947		(85)	\$ 880,961
Maternal And Child Health Services Block Grant To The States	93.994	000013344	USC			11,945	\$ 228,154
Maternal And Child Health Services Block Grant To The States	93.994	000015557				216,209	\$ 228,154
Total Department Of Health And Human Services						\$ 551,229	\$ 11,340,499
Department Of Justice:							
Crime Victim Assistance	16.575	000013880	ETR ASSOCIATES	HX21201185		\$ 14,923	\$ 389,319
Crime Victim Assistance	16.575	000014834		HX22 21 1185	10,833	5,452	\$ 389,319
Crime Victim Assistance	16.575	000015577		HX24 23 1185	92,211	381,817	\$ 389,319
Crime Victim Assistance	16.575	SGG012501				(12,873)	\$ 389,319
Total Department Of Justice						\$ 103,044	\$ 389,319
TOTAL PROGRAMS NOT CLUSTERED:						\$ 654,273	\$ 11,729,818
TOTAL EXPENDITURES OF FEDERAL AWARDS						\$ 20,713,177	\$ 68,982,462

CHILDREN’S HOSPITAL LOS ANGELES

NOTES TO THE SCHEDULE OF EXPENDITURES OF FEDERAL AWARDS FOR THE YEAR ENDED JUNE 30, 2024

1. BASIS OF PRESENTATION

The schedule of expenditures of federal awards includes federal grant activity under federal government programs of Children’s Hospital of Los Angeles (“Hospital”) for the year ended June 30, 2024. The information in this schedule is presented in accordance with the requirements of Title 2 US *Code of Federal Regulations Part 200, Uniform Administrative Requirements, Cost Principles, and Audit Requirements for Federal Awards*. Therefore, some amounts presented in this schedule may differ from amounts presented in, or used in the preparation, of the general-purpose financial statements.

2. EQUIPMENT EXPENDITURES

All capital expenditures are charged in the period in which the expenditure is incurred or as otherwise determined appropriate by and negotiated with the awarding agency.

3. PASS-THROUGH AWARDS

The Hospital receives certain federal awards from pass-through agencies where the amounts are commingled by the pass-through agency with other nonfederal funds and cannot be separately identified. The total amount of pass-through awards is included in the schedule of expenditures of federal awards.

4. SUBRECIPIENTS

The Hospital also provided federal awards to subrecipients, of which the amounts are presented in the schedule of expenditures of federal awards.

5. INDIRECT RATE

The Hospital has not elected to use 10% de minimis indirect cost rate.

6. CREDIT BALANCES

Net credit balances in the expenditures column are the result of timing differences from effective dates of awards to when the Hospital receives award notices, delays in receiving no-cost extensions, and supplemental expenditures and do not represent overbillings or overpayments on such awards. As a result of these circumstances, total federal expenditures of \$ 68,982,462 is shown net of \$ 717,676 of credit balances for the year ended June 30, 2024.

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INDEPENDENT AUDITOR'S REPORT ON INTERNAL CONTROL OVER FINANCIAL REPORTING AND ON COMPLIANCE AND OTHER MATTERS BASED ON AN AUDIT OF FINANCIAL STATEMENTS PERFORMED IN ACCORDANCE WITH *GOVERNMENT AUDITING STANDARDS*

To the Board of Directors of
Children's Hospital Los Angeles:

We have audited, in accordance with the auditing standards generally accepted in the United States of America and the standards applicable to financial audits contained in *Government Auditing Standards* issued by the Comptroller General of the United States (*Government Auditing Standards*), the financial statements of Children's Hospital Los Angeles and its subsidiaries (the "Hospital"), which comprise the consolidated balance sheet as of June 30, 2024, and the related consolidated statements of activities, changes in net assets, and cash flows for the year then ended, and the related notes to the consolidated financial statements (collectively referred to as the "financial statements"), and have issued our report thereon dated November 18, 2024.

Report on Internal Control over Financial Reporting

In planning and performing our audit of the financial statements, we considered the Hospital's internal control over financial reporting ("internal control") as a basis for designing audit procedures that are appropriate in the circumstances for the purpose of expressing our opinion on the financial statements, but not for the purpose of expressing an opinion on the effectiveness of the Hospital's internal control. Accordingly, we do not express an opinion on the effectiveness of the Hospital's internal control.

A deficiency in internal control exists when the design or operation of a control does not allow management or employees, in the normal course of performing their assigned functions, to prevent, or detect and correct, misstatements on a timely basis. A *material weakness* is a deficiency, or a combination of deficiencies, in internal control, such that there is a reasonable possibility that a material misstatement of the Hospital's financial statements will not be prevented or detected and corrected on a timely basis. A *significant deficiency* is a deficiency, or a combination of deficiencies, in internal control that is less severe than a material weakness, yet important enough to merit attention by those charged with governance.

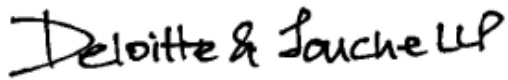
Our consideration of internal control was for the limited purpose described in the first paragraph of this section and was not designed to identify all deficiencies in internal control that might be material weaknesses or significant deficiencies. Given these limitations, during our audit we did not identify any deficiencies in internal control that we consider to be material weaknesses. However, material weaknesses or significant deficiencies may exist that were not identified. Given these limitations, during our audit we did not identify any deficiencies in internal control that we consider to be material weaknesses. We identified certain deficiencies in internal control, described in the accompanying Schedule of Findings and Questioned Costs in Item 2024-01 that we consider to be a significant deficiency.

Report on Compliance and Other Matters

As part of obtaining reasonable assurance about whether the Hospital's financial statements are free from material misstatement, we performed tests of their compliance with certain provisions of laws, regulations, contracts, and grant agreements, noncompliance with which could have a direct and material effect on the determination of financial statements. However, providing an opinion on compliance with those provisions was not an objective of our audit, and accordingly, we do not express such an opinion. The results of our tests disclosed no instances of noncompliance or other matters that are required to be reported under *Government Auditing Standards*.

Purpose of this Report

The purpose of this report is solely to describe the scope of our testing of internal control and compliance and the results of that testing, and not to provide an opinion on the effectiveness of the Hospital's internal control or on compliance. This report is an integral part of an audit performed in accordance with *Government Auditing Standards* in considering the Hospital's internal control and compliance. Accordingly, this communication is not suitable for any other purpose.

A handwritten signature in black ink that reads "Deloitte & Touche LLP". The signature is written in a cursive, flowing style.

November 18, 2024

INDEPENDENT AUDITOR'S REPORT ON COMPLIANCE FOR EACH MAJOR FEDERAL PROGRAM AND REPORT ON INTERNAL CONTROL OVER COMPLIANCE

To the Board of Directors of
Children's Hospital Los Angeles:

Report on Compliance for Each Major Federal Program

Opinion on Each Major Federal Program

We have audited Children's Hospital Los Angeles (the "Hospital") compliance with the types of compliance requirements described in the *OMB Compliance Supplement* that could have a direct and material effect on each of the Hospital's major federal programs for the year ended June 30, 2024. The Hospital's major federal programs are identified in the summary of auditor's results section of the accompanying schedule of findings and questioned costs.

In our opinion, the Hospital complied, in all material respects, with the compliance requirements referred to above that could have a direct and material effect on each of its major federal programs for the year ended June 30, 2024.

Basis for Opinion on Each Major Federal Program

We conducted our audit of compliance in accordance with auditing standards generally accepted in the United States of America (GAAS); the standards applicable to financial audits contained in *Government Auditing Standards* issued by the Comptroller General of the United States (*Government Auditing Standards*); and the audit requirements of Title 2 U.S. *Code of Federal Regulations* Part 200, *Uniform Administrative Requirements, Cost Principles, and Audit Requirements for Federal Awards* (Uniform Guidance). Our responsibilities under those standards and the Uniform Guidance are further described in the Auditor's Responsibilities for the Audit of Compliance section of our report.

We are required to be independent of the Hospital and to meet our other ethical responsibilities, in accordance with relevant ethical requirements relating to our audit. We believe that the audit evidence we have obtained is sufficient and appropriate to provide a basis for our opinion on compliance for each major federal program. Our audit does not provide a legal determination of Hospital's compliance with the compliance requirements referred to above.

Responsibilities of Management for Compliance

Management is responsible for compliance with the requirements referred to above and for the design, implementation, and maintenance of effective internal control over compliance with the requirements of laws, statutes, regulations, rules and provisions of contracts or grant agreements applicable to Hospital's federal programs.

Auditor's Responsibilities for the Audit of Compliance

Our objectives are to obtain reasonable assurance about whether material noncompliance with the compliance requirements referred to above occurred, whether due to fraud or error, and express an

opinion on Hospital's compliance based on our audit. Reasonable assurance is a high level of assurance but is not absolute assurance and therefore is not a guarantee that an audit conducted in accordance with GAAS, *Government Auditing Standards*, and the Uniform Guidance will always detect material noncompliance when it exists. The risk of not detecting material noncompliance resulting from fraud is higher than for that resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control. Noncompliance with the compliance requirements referred to above is considered material, if there is a substantial likelihood that, individually or in the aggregate, it would influence the judgment made by a reasonable user of the report on compliance about Hospital's compliance with the requirements of each major federal program as a whole.

In performing an audit in accordance with GAAS, *Government Auditing Standards*, and the Uniform Guidance, we

- exercise professional judgment and maintain professional skepticism throughout the audit.
- identify and assess the risks of material noncompliance, whether due to fraud or error, and design and perform audit procedures responsive to those risks. Such procedures include examining, on a test basis, evidence regarding Hospital's compliance with the compliance requirements referred to above and performing such other procedures as we considered necessary in the circumstances.
- obtain an understanding of Hospital's internal control over compliance relevant to the audit in order to design audit procedures that are appropriate in the circumstances and to test and report on internal control over compliance in accordance with the Uniform Guidance, but not for the purpose of expressing an opinion on the effectiveness of Hospital's internal control over compliance. Accordingly, no such opinion is expressed.

We are required to communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit and any significant deficiencies and material weaknesses in internal control over compliance that we identified during the audit.

Report on Internal Control Over Compliance

A deficiency in internal control over compliance exists when the design or operation of a control over compliance does not allow management or employees, in the normal course of performing their assigned functions, to prevent, or detect and correct, noncompliance with a type of compliance requirement of a federal program on a timely basis. A *material weakness in internal control over compliance* is a deficiency, or a combination of deficiencies, in internal control over compliance, such that there is a reasonable possibility that material noncompliance with a type of compliance requirement of a federal program will not be prevented, or detected and corrected, on a timely basis. A *significant deficiency in internal control over compliance* is a deficiency, or a combination of deficiencies, in internal control over compliance with a type of compliance requirement of a federal program that is less severe than a material weakness in internal control over compliance, yet important enough to merit attention by those charged with governance.

Our consideration of internal control over compliance was for the limited purpose described in the Auditor's Responsibilities for the Audit of Compliance section above and was not designed to identify all deficiencies in internal control over compliance that might be material weaknesses or significant deficiencies in internal control over compliance. Given these limitations, during our audit we did not identify any deficiencies in internal control over compliance that we consider to be material

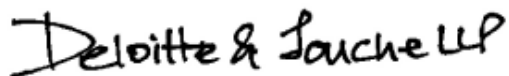
weaknesses, as defined above. However, material weaknesses or significant deficiencies in internal control over compliance may exist that were not identified.

Our audit was not designed for the purpose of expressing an opinion on the effectiveness of internal control over compliance. Accordingly, no such opinion is expressed.

The purpose of this report on internal control over compliance is solely to describe the scope of our testing of internal control over compliance and the results of that testing based on the requirements of the Uniform Guidance. Accordingly, this report is not suitable for any other purpose.

Report on Schedule of Expenditures of Federal Awards Required by the Uniform Guidance

We have audited the financial statements of the Hospital as of and for the year ended June 30, 2024, and have issued our report thereon dated November 18, 2024, which contained an unmodified opinion on those financial statements. Our audit was performed for the purpose of forming an opinion on the financial statements as a whole. The accompanying schedule of expenditures of federal awards is presented for purposes of additional analysis as required by the Uniform Guidance and is not a required part of the financial statements. Such information is the responsibility of management and was derived from and relates directly to the underlying accounting and other records used to prepare the financial statements. The information has been subjected to the auditing procedures applied in the audit of the financial statements and certain additional procedures, including comparing and reconciling such information directly to the underlying accounting and other records used to prepare the financial statements or to the financial statements themselves, and other additional procedures in accordance with auditing standards generally accepted in the United States of America. In our opinion, the schedule of expenditures of federal awards is fairly stated in all material respects in relation to the financial statements as a whole.

A handwritten signature in black ink that reads "Deloitte & Touche LLP". The signature is written in a cursive, flowing style.

November 20, 2024

CHILDREN'S HOSPITAL LOS ANGELES

SCHEDULE OF FINDINGS AND QUESTIONED COSTS FOR THE YEAR ENDED JUNE 30, 2024

SECTION I—SUMMARY OF AUDITOR'S RESULTS

Consolidated Financial Statements

Type of auditor's report issued: Unqualified

Internal control over financial reporting:

- Material weakness(es) identified? ☐ Yes ☒ No
- Significant deficiency(ies) that are not considered to be material weaknesses? ☒ Yes ☐ None reported
- Noncompliance material to financial statements noted? ☐ Yes ☒ No

Federal Awards

Internal control over major programs:

- Material weakness(es) identified? ☐ Yes ☒ No
- Significant deficiency(ies) that are not considered to be material weaknesses? ☐ Yes ☒ None reported

Type of auditor's report issued on compliance for major programs: Unqualified

Any audit findings disclosed that are required to be reported in accordance with 2 CFR 200.516(a)? ☐ Yes ☒ No

Identification of Major Programs:

Assistance Listing Number

Various
93.493

Name of Federal Program or Cluster

Research and Development Cluster
Congressional Directives

Dollar threshold used to distinguish between Type A and Type B programs

\$2,069,474

- Auditee qualified as low risk auditee ☒ Yes ☐ No

SECTION II—FINANCIAL STATEMENT FINDINGS

Finding 2024-01 – Significant deficiency in internal control over financial reporting

Criteria – Internal controls over financial reporting should be designed and operating in a manner that allows management or employees, in the normal course of performing their assigned functions, to prevent, or detect and correct, misstatements on a timely basis.

Condition – The review of contractual analysis and its related reconciliation, as well as the review of timely processing of credit balances were not performed at a sufficient precision level. The aggregation of these controls resulted in a significant control deficiency.

Cause – The review as designed did not take into account patient account fluctuations that could result in data anomalies with the contractual reserving process. The changes in patient account receivable collection timing during the year highlighted a need to apply a narrower threshold over the review of the manual reserves and the related reconciliation. Also, additional resources were needed to timely process credit balances, which have now been diverted to this operation.

Effect and Context– There was an identified adjustment related to an immaterial reclassification of a patient account refund obligation and true-up of contractual reserves and provision balance as a result.

Recommendation – Until systematic changes are made to capture automatic contractual adjustments correctly, we recommend that 1) threshold over manual review of patient account data should be lowered to increase efforts to identify anomalies and inconsistencies that may exist in the contractual provision application in patient accounts, 2) additional scrutiny should be performed on contractual reserve reconciliations to identify classification of reconciling adjustments and 3) resources should be redirected to bring down volumes of historic credit balances and root cause analysis should be performed to identify refunds and to assess impact to the manual reserving process.

Questioned Cost – N/A

Views of Responsible Officials – See Corrective Action Plan.

SECTION III—FEDERAL AWARD FINDINGS AND QUESTIONED COSTS

None noted.

* * * * *



Corrective Action Plan

Finding Reference Number 2024-01 - Significant deficiency in internal control over financial reporting

View of Responsible Officials

Management agrees with the comment. The determination of contractual allowances is completed systematically on accounts once they are billed to the payor. The contracts used to calculate the expected reimbursement are complicated, and subject to interpretation by both hospital and the payor. As differences in calculations are noted, the hospital reviews the system calculations and adjusts the parameters or, when appropriate, re-bill or appeal the claims. We have various controls including a review of open account balances to identify payor and systemic variances as well as management's review of the liquidation schedules on a monthly basis which allows us to adjust certain aspects of the contractual allowance model to reflect current payment trends. We have also refocused resources to continue to bring down patient account credit balances.

We plan to continuously review our existing processes and internal controls to identify opportunities for improvement in light of the identified errors. We further plan to reduce our threshold to analyze a wider patient account population that may be reflecting below average contractual rates versus focusing on accounts where no adjustments are applied. We also plan to timely review manual adjustments posted to the contractual reserve as part of our account reconciliations and continue to deploy resources to bring down credits embedded in receivables to process potential patient refunds.

Anticipated Completion Date: December 31, 2024

Contact Officials – Scott Lieberenz, Chief Financial Officer